

WHERE THE MATERIAL SITS AGAINST THE THREE MATTERS IN ISSUE

Cory Shepherd · WC/2024/227 · 4 September 2026. The three matters are those written on the Notice of Non-Party Disclosure requested by the Workers' Compensation Regulator and sealed 4 July 2025, which is attached to my email separately. Every document referred to below is in this file, complete and unaltered. **The page references are clickable**, and the bookmarks panel lists each document.

This page is a finding aid only. It makes no submission about what any item shows, and nothing in it is intended to bear on your opinion. It contains nothing that was not provided with my letter of 17 August 2026, other than the page references.

PLEASE NOTE — WHAT IS NOT ASKED. The letter of instruction of 17 August 2026 (in this file at [p. 3](#)) asks a longer set of questions. Only the five questions in my email of 4 September 2026 are asked now. In particular, **questions 3.4 to 3.8 of that letter — capacity, restrictions and adjustments — are NOT asked for this purpose and should be set aside.** They are the employer's questions, they are not urgent, and they can be dealt with separately.

MATTER 1 — DID MR SHEPHERD SUSTAIN A PERSONAL INJURY?

Material	Where
Your own clinical file for me, from 24 October 2024	held by the practice
Your report of 13 February 2025	held by the practice
General-practice records, Our Medical Ashmore — including the entries of 26.10.2022 ("ADHD", "Anxiety", in the Past Medical History list of the referral of 16.05.2024); 16.11.2023 (Dr Nanayakkara); 16.05.2024 (renewal of psychiatric referral); 28.06.2024 (Dr Slawinski); 01.07.2024 (Dr Hawes)	Attachment 4a, p. 34 · schedule E, p. 10
Workers' compensation medical certificate of Dr Hawes, signed 8 September 2024	Attachment 4b, p. 48
Employee Capability Checklist, 3 July 2026	Attachment 4c, p. 50
The Regulator accepts that your report of 13 February 2025 states Major Depressive Disorder — as what the report says; its accuracy is not accepted	schedule A item 38, p. 10
The Regulator does not accept the accuracy of the entry of 16 November 2023, and asserts a past medical history of anxiety and ADHD from 26 October 2022	schedule B items 34, 35, p. 10

MATTER 2 — DID THE INJURY ARISE OUT OF, OR IN THE COURSE OF, THE EMPLOYMENT?

Rows are grouped to the three sources of accumulation described in question 3 of my email of 4 September 2026 — **(a)** fatigue from the shift events, **(b)** further rostering errors, **(c)** the continuing need to follow up unresolved issues. Rows marked — are context.

Material	Where
— The requirements of the position as the employer describes them — continuous shift work over the full 24-hour period, 7 days a week; the Emergency Response process "strictly adhering to protocols and timeframes"; "maintain call queues to minimum at all times"; multitasking under "high volume call traffic"; judgement "in situations where precedence have not been set and procedures not defined"; "limited supervision"	Attachment 3, p. 30
(a) Accepted: on 17–18 March 2024 rostered to finish at 23:00 and commence at 06:00 — a break of 7 hours	schedule A item 1, p. 10
(a) Accepted: the fatigue policy and the Award require a minimum of 10 hours, or 8 hours by written agreement	schedule A item 3
(a) The Regulator's document of 13 May 2026: "only a 7-hour break (rather than an 8-hour break) ... a result of human error and not intentional or repeated"; leave taken 19 March 2024, paid	schedule A2 22(a), 22(c)
(a) Recorded by the Chief Executive: the requested fatigue risk management training records and register entries do not exist, such training applying only to health practitioners and clinical assistants; fatigue risk management assessment at that Switchboard was implemented only after 30 June 2024	Attachment 2, p. 24
(a) Recorded by the Chief Executive: a spreadsheet of recorded MET calls is available for the period 17–18 March 2024	schedule C items 1–2
(b) Accepted: Ms Reese, 7 August 2023 — "a rostering error that was accidentally made by Chloe with regards to night shifts"	schedule A item 5
(b) Accepted: payroll instruction to correct the shifts issued 3 May 2024; "I am waiting payroll confirmation" 21 May 2024; correction submitted 28 May 2024	schedule A items 40, 46, 41
(c) Recorded by the Chief Executive: employee complaints about Switchboard operational errors are "made directly to the Line Manager" and "managed solely via email or verbally with the complainant"	schedule C item 3(a)
(c) The Regulator's document of 13 May 2026 on the leave of 20–27 February 2024: "a review indicates that in fact, the attachments were present"; "a matter of human error"	schedule A2 14(e), 14(f)
(b) and (c) The three stressors identified by the employer in the Employee Capability Checklist of 3 July 2026 — complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering	Attachment 4c, p. 50
— Accepted: maintaining accurate contact details for medical staff is "a critical function of the Switchboard to ensure effective clinical handover and patient safety"	schedule A item 8
— Changes to working hours approved during 2026 by Mr Hughes as delegate — 27 February, 17 April, 9 June	Attachment 5, p. 55

MATTER 3 — WAS THE EMPLOYMENT A SIGNIFICANT CONTRIBUTING FACTOR TO THE INJURY?

Material	Where
All of the material listed above	Matters 1 and 2
Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement	held by the practice
Your clinical relationship with me began on 24 October 2024, and my partner attended that consultation with me. Each of those matters arose while I was under your care and is recorded in your own file	held by the practice
The sequence, from sources other than my account: onset pleaded at or about 18 June 2024 · application for compensation lodged 1 July 2024 · rejected 13 September 2024 · correspondence as to abandonment 8 October 2024 · review decision 24 October 2024 · your report 13 February 2025 · reinstatement effective 20 September 2024, agreement executed 21 February 2025 · Employee Capability Checklist 3 July 2026	schedules A, B, C · schedule
My letter of instruction of 17 August 2026, and the five questions in my email of 4 September 2026	Letter, p. 3

CHRONOLOGY — FOR EASE OF REFERENCE ONLY, NOT A SUBMISSION

Cory Shepherd, date of birth 11 January 1991 · WC/2024/227 · prepared 4 September 2026. This page records dated events and where each is recorded. It draws no conclusion from any of them, and it is not intended to bear on your opinion. Where the record is unclear or a date is not established, that is said so in terms rather than estimated.

Date	Event	Where it is recorded
26 Oct 2022	General-practice record. "ADHD" and "Anxiety" appear in the Past Medical History list carried in the referral of 16 May 2024.	GP records, Our Medical Ashmore
16 Nov 2023	General-practice consultation, Dr Nanayakkara.	GP records
14 Jan 2024	A family matter concerning my sister, then 15, attended at police direction. Not an intimate-partner matter. My line manager was told that morning.	My account; manager's reply in the record
20–27 Feb 2024	Period of leave, later the subject of a dispute about whether attachments had been supplied. The Respondent now records "a review indicates that in fact, the attachments were present" and "a matter of human error".	Respondent's document, 13 May 2026, 14(e), 14(f)
17–18 Mar 2024	Rostered to finish at 23:00 and to commence at 06:00 — a break of 7 hours. The fatigue policy and the Award require a minimum of 10 hours, or 8 by written agreement. MET calls were recorded over that period.	Accepted — schedule A items 1 and 3; Chief Executive's letter, 5 Jun 2026
19 Mar 2024	Leave taken, paid.	Accepted — Respondent's document, 22(c)
15 Apr 2024	A change to Switchboard working arrangements communicated to operators by email. The Respondent lists no document recording consultation before it.	Respondent's amended List of Documents, 14 Aug 2026
3 May 2024	Payroll instructed to correct the shifts.	Accepted — schedule A item 40
16 May 2024	General practice renewed the psychiatric referral.	GP records
21 May 2024	"I am waiting payroll confirmation."	Accepted — schedule A item 46
28 May 2024	The correction was submitted — 8 weeks after the shifts.	Accepted — schedule A item 41
18 Jun 2024	Onset of the injury, as pleaded in the appeal.	Form 9A, as pleaded
28 Jun 2024	General-practice consultation, Dr Slawinski.	GP records
30 Jun 2024	Fatigue risk management assessment was implemented at Logan Hospital Switchboard only after this date. No fatigue risk management training records or register entries exist for the position, such training applying only to health practitioners and clinical assistants.	Chief Executive's letter, 5 Jun 2026
1 Jul 2024	General-practice consultation, Dr Hawes. Application for compensation lodged with WorkCover Queensland.	GP records; WorkCover file
12 Jul 2024	"Event overview" provided by me to WorkCover Queensland.	Respondent's List of Documents, item 12
8 Sep 2024	Workers' compensation medical certificate signed by Dr Hawes.	Certificate
13 Sep 2024	WorkCover Queensland rejected the application for compensation.	WorkCover reasons, 13 Sep 2024
20 Sep 2024	Separation date nominated by the employer in the agreement later executed.	Deed, recital D
late Sep / early Oct 2024	My grandfather died. Exact date to be confirmed — see the note at the foot.	To be confirmed
9 Oct 2024	Correspondence from the Acting Executive Director treating the employment as abandoned was forwarded to me at 4:10 pm. I replied in writing.	Employer correspondence, 9 Oct 2024
24 Oct 2024	First consultation with Dr Krishnaiah. My partner attended that consultation with me. The Regulator's review decision issued the same day.	Your file; Review Decision 69983
13 Feb 2025	Dr Krishnaiah's report to QSuper. It records workplace stress arising from management and rostering, and refers to relationship breakdown, job loss and bereavement.	Your file
Feb – Mar 2025	My relationship with my partner ended.	My account
21 Feb 2025	The agreement with the employer was executed.	Deed
3 Jul 2026	Employee Capability Checklist completed by the employer. It identifies three stressors for the position — complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering.	Employee Capability Checklist

MATTERS NOT ESTABLISHED, AND MATTERS I CANNOT DATE

Item	Position
The date of my grandfather's death	Not established from any document I hold. It was in or about the period covered by the leave immediately before the correspondence of 9 October 2024. I have not estimated it. If it matters to your answer, it may be in your own file or the general-practice record.
Any psychological diagnosis before 24 October 2024	No psychological or psychiatric diagnosis is recorded in the material I hold as at any date before 24 October 2024. The general-practice record of 26 October 2022 lists "ADHD" and "Anxiety" in a past medical history; I make no assertion about what that entry represents.
The separation date	The documents differ. The agreement records 20 September 2024; an earlier filing of mine records 8 October 2024; the correspondence treating the employment as abandoned was sent on 9 October 2024. All three fall after 18 June 2024. I have not resolved the difference and nothing here depends on it.
Physical health matters arising later	Matters arising in 2026 are not included here. They are not within the three questions and I have not asked you to address them.
The order of events after the onset	The relationship ending, the loss of the job and the bereavement are each recorded above at the date the record gives. I draw no conclusion from the order and leave that entirely to you.

Letter of instruction — psychiatric report

Dr Ravikumar Bangalore Krishnaiah, Consultant Psychiatrist, Mind and Memory Service

From: Cory Lea Shepherd · 17 August 2026

In answer to the request for medical information made by Metro South Hospital and Health Service on 31 July 2026, and addressing the matters in issue in WC/2024/227, Queensland Industrial Relations Commission

On 31 July 2026, Metro South Hospital and Health Service requested medical information about my condition and my capacity for work. That request is signed by **Mr Scott Hughes, Director, Corporate Services, Logan Beaudesert Health Service**. It is enclosed as Attachment 7, including the nine-question schedule. This letter instructs the report that answers that request. The nine questions are reproduced in the appendix, in its own terms, with the question that answers each.

Your practice has been served with a Form 29 Notice of Non-Party Disclosure requested by the Workers' Compensation Regulator. It is signed by **Ms Renee Matheson, Senior Appeals Officer**, sealed **4 July 2025**, and enclosed as Attachment 6. The three matters written on that Form 29 are answered at 1, 2 and 3. One report answers the Form 29 and the Health Service request.

PART A — THE ATTACHMENTS

Seven documents accompany this letter. I ask you to reason from all of them, as set out in Part B.

No.	Document	Author	Status
1	Form 24 — Respondent's response to the notice to admit facts, signed 18 February 2026 by Ms Renee Matheson , Senior Appeals Officer. The notice is included for the item numbers.	Ms Renee Matheson Senior Appeals Officer Workers' Compensation Regulator	Assumed fact (what the response admits)
2	Letter of Ms Noelle Cridland , Chief Executive, Metro South Hospital and Health Service, 5 June 2026 (ref K-LM26/729), to the Commission.	Ms Noelle Cridland Chief Executive Metro South Hospital and Health Service	Assumed fact
3	Role description — Administration Officer, Switchboard Services (AO3), Logan Hospital.	The employer	Assumed fact
4	The clinical record — your own file for me from 24 October 2024; my general-practice records (Our Medical Ashmore); the workers' compensation medical certificates; the employee capability checklist of 3 July 2026.	Clinicians	Clinical record (not assumed fact)
5	Approved changes to working hours, 2026 — three forms of my employer. Each was approved by Mr Scott Hughes, Director, Corporate Services , as the authorised delegate (27 February, 17 April and 9 June 2026).	Mr Scott Hughes Director, Corporate Services (delegate)	Assumed fact
6	Form 29 — Notice of Non-Party Disclosure served on this practice, sealed 4 July 2025 . Requested by the Workers' Compensation Regulator. Signed by Ms Renee Matheson , Senior Appeals Officer. Nominated party: the Practice Manager, Mind and Memory Service. The three matters in issue are written on that Form 29.	Ms Renee Matheson Senior Appeals Officer Workers' Compensation Regulator	The questions (not assumed fact)
7	Request for medical information, 31 July 2026 — letter to the employee and the nine-question schedule, both signed by Mr Scott Hughes, Director, Corporate Services . The schedule is the employer's request. Questions 3 and 9 on that schedule are workplace questions and are not asked of you.	Mr Scott Hughes Director, Corporate Services	The employer's questions (not assumed fact)

Also enclosed: a **schedule of assumed facts**. It identifies, by source and paragraph, the facts you are asked to assume, and records what the Regulator does not accept. Attachment 1 does not include items 26 to 31 of the list it responds to.

PART B — THE BASIS ON WHICH I ASK YOU TO WRITE

I ask you to reason from the following, and from nothing else in relation to the workplace.

	Reason from	Where
1	The facts admitted by the Workers' Compensation Regulator in the Form 24 response signed by Ms Renee Matheson, Senior Appeals Officer, on 18 February 2026	Attachment 1 schedule A, A2, B

	Reason from	Where
2	What is written on the letter of Ms Noelle Cridland , Chief Executive, Metro South Hospital and Health Service, 5 June 2026 (ref K-LM26/729)	Attachment 2 schedule C
3	The requirements of the position, as my employer describes them	Attachment 3 schedule D
4	The clinical records, in full — your own file, my general-practice records and the certificates	Attachment 4 schedule E
5	The changes to my working hours approved during 2026 by Mr Scott Hughes as delegate	Attachment 5 schedule F
	And take into account	Asked at
6	The relationship breakdown and the other life stressors recorded in your own file	question 3.1
7	The past medical history recorded in the general-practice records	question 1.3
8	The three matters written on the Form 29 signed by Ms Renee Matheson (Attachment 6)	Matters 1, 2 and 3
9	The nine questions written on the request signed by Mr Scott Hughes on 31 July 2026 (Attachment 7)	Appendix; 3.4 to 3.8

Items 1, 2, 3 and 5 were written by the other parties — Ms Matheson, Ms Cridland, the employer, and Mr Hughes. I ask you to assume the facts they record. **My own account of the workplace is provided as clinical context only, and is not the basis on which I ask you to reason.** Where your history from me differs from any fact in Attachments 1 to 3 or 5, please say so expressly rather than resolve the difference.

The schedule also records the matters the Regulator does not accept. Please do not treat any of those as established. They are set out so that you have the position complete rather than one side of it.

Please do not adopt, or treat as established, any conclusion, characterisation or finding of any other decision-maker. **Your opinion is yours alone.**

The sequence, from a source other than my account. The following dates are taken from the Regulator's amended List of Documents dated 14 August 2026, by its item number.

Item	Date	Document
46	1 July 2024	Text, WorkCover to me, noting the claim registered
7	1 July 2024 11 August 2024 8 September 2024	Workers' compensation medical certificates — Dr Peter Hawes
8	7 August 2024	Workers' compensation medical certificate — Dr Ki Pang
2	13 September 2024	WorkCover Queensland reasons for decision
3	16 September 2024	Application for review
9	24 October 2024	Email, Dr Krishnaiah — noting injury and medication
4	24 October 2024	Review Unit reasons for decision
5	26 November 2024	Notice of Appeal
10	13 February 2025	Report of Mind and Memory Service
11	Various	Practice records — Our Medical Ashmore

The clinical records are the complete records held by the Regulator in this matter, provided in full and unedited — including entries unrelated to this matter and entries that may not assist me. I have not selected among them. If any further records or other material would assist you, please say so and I will obtain them.

PART B2 — WHAT IS ALREADY RECORDED, AND WHAT I AM NOT ASKING YOU TO ESTABLISH

Attachments 1 to 3 and 5 already record the nature of the work, what occurred, and what the employer says was and was not in place. **You are not asked to find any of that, and you are not asked to take any of it from me.**

Already recorded, by the Regulator or by my employer: the requirements of the position · the accuracy of Switchboard information as critical to clinical handover and patient safety · the applicable minimum break and the 7-hour break in fact rostered on 17 to 18 March 2024, described as human error · the related administrative sequence in rostering, leave and pay · that no fatigue risk assessment applied to the position, fatigue risk management being implemented only after 30 June 2024, with no consequential changes to operating procedures ·

and the changes to my working hours approved during 2026. **The enclosed schedule sets each out by source; you are not asked to find any of them.**

One further matter, provided for completeness. In May 2024 I made a complaint concerning the conduct of my line manager and the Director. On **24 December 2024** the Ethical Standards Unit determined that the complaint **constituted a Public Interest Disclosure**. That fact is accepted by the Workers' Compensation Regulator. I provide it as part of the occupational history and for no other purpose. I do not ask you to express any view on the merits of the complaint, on anything that followed it, or on any connection between it and any later event. **If it does not bear on your clinical opinion, please say so.**

What remains, and what I do ask you, is the clinical question: whether exposure of that kind bears on the condition you have diagnosed.

PART C — THE QUESTIONS

The Health Service's nine questions are answered within the structure below, at the places shown in the appendix. Written on the Form 29 signed by Ms Renee Matheson (Attachment 6):

- "1. Did Mr Shepherd sustain a personal injury*
- 2. Did the personal injury arise out of or in the course of Mr Shepherd's employment*
- 3. Was Mr Shepherd's employment a significant contributing factor to the injury"*

The questions below are arranged so that one report answers both. The employer's questions on capacity, restrictions and adjustments are taken up at **3.4 to 3.8**; the three matters on her Form 29 are addressed at **1, 2 and 3**. I ask you to take them in the order given.

MATTER 1 — DID MR SHEPHERD SUSTAIN A PERSONAL INJURY?

1.1 The diagnosis; the classificatory framework applied and the code; the specific diagnostic criteria met and when each was met; and the date of onset as best the clinical record establishes it.

1.2 The differential diagnoses considered, and your reasons for including or excluding each.

1.3 My background. The records include the following entries, which I ask you to address. They are set out so that they are before you; **I make no submission about what any of them shows.**

26/10/2022 — "ADHD" and "Anxiety", appearing in the "Past Medical History" list of the referral letter of 16 May 2024 from Dr William Zhao to Dr Arash Amini, together with "28/06/2023 Infection".

16/11/2023 — Dr Priyal De Silva Nanayakkara: "Poor sleep. Shift work. Takes melatonin 2 mg to help to go to sleep, soem time 4 mg, but does not help much. Cannot work/ do shifts if he does not get a good sleep. Does not take them every day. No psychological illness such as depression/ pyscosis. mood good." — melatonin and temazepam prescribed.

16/05/2024 — the consultation note records "renew referral to psychiatrist", and the referral letter above is that referral, "for ongoing care and management"; together with the current medications recorded in it.

28/06/2024 — Dr Bogdan Slawinski: "wants melatonin, to help to sleep ... stress at work ... upset by people not following rules"; reason for visit "Anxiety".

01/07/2024 — Dr Peter Hawes: "been there 5 years"; reason for visit "work stress".

On those entries and on the material generally, I ask:

(a) what each of those entries records; and **whether any diagnosis of an anxiety disorder appears anywhere in the material**;

(b) what, if anything, the current medications recorded in the letter of 16 May 2024 indicate as to treatment at that date;

(c) how, if at all, attention deficit features bear on vulnerability to injury where continuous shift work is worked without adequate recovery between shifts, and under sustained high-volume concurrent demand of the kind described at Attachment 3;

(d) what significance you attach to the **renewal of referral on 16 May 2024**, having regard to the entry of 16 November 2023; what, if anything, the material shows occurred between those two dates; and when psychiatric care began, and for what;

(e) what the material shows as to any **psychiatric treatment, or any absence from work on psychological grounds**, before 2024 — and what that indicates about my level of function in this role, having worked continuous shift work in this position from 2019. Does anything in the records bear on the date of onset?

(f) whether there was any pre-existing condition or vulnerability and, if so, whether the matters at Attachments 1 to 3 **aggravated** it, and to what extent.

1.4 Whether any premorbid personality features bear on the diagnosis, and if so how — from your own current clinical assessment. I do not ask you to adopt or repeat any earlier characterisation.

1.5 A complete list of every document you reviewed and every person you saw, with the dates and duration of each examination.

MATTER 2 — DID THE INJURY ARISE OUT OF, OR IN THE COURSE OF, THE EMPLOYMENT?

2.1 The conditions of the work. Attachment 3 records that the position requires continuous shift work across the full 24-hour period, seven days a week; participation in the Emergency Response process, distributing emergency notifications in accordance with code procedures and “strictly adhering to protocols and timeframes”; “maintaining call queues to minimum at all times”; multitasking and operating under pressure with “high volume call traffic”; exercising judgement “in situations where precedence have not been set and procedures not defined”; and working with “limited supervision”. Attachment 1 records that the accuracy of Switchboard information is **critical to clinical handover and patient safety**.

From a clinical perspective, whether and how conditions of that kind — the level of demand, the degree of control, and the adequacy of support — can contribute to a condition of the kind you have diagnosed.

2.2 Fatigue, and the rostering of 17 to 18 March 2024. Attachment 1 records that on those dates I was rostered to finish at 23:00 and to commence the next shift at 06:00 — **a break of 7 hours**. The applicable minimum is recorded as **10 hours, or 8 hours by written agreement**. A further document of the Regulator, dated 13 May 2026 — the passages are set out in the schedule at A2 — records that an 8-hour written agreement applied and describes the shift as separated “by only a 7-hour break (rather than an 8-hour break) ... a result of human error”. **On either figure the break was shorter than the minimum**. Attachment 2 records that emergency (MET) calls were recorded on those shifts, and that leave was taken on 19 March 2024. Attachment 2 also records that **no fatigue risk assessment applied to the position**, and that fatigue risk management was implemented at that Switchboard only after 30 June 2024.

What clinical significance, if any, do you attach to that sequence — and does fatigue arising from a shortened break resolve with the break itself, or can it carry forward? Separately: does working continuously in a role of this kind where no fatigue risk assessment or monitoring is in place differ, in terms of the risk of psychological injury, from working in the same role where such measures are in place?

2.3 The administrative sequence. Attachment 1 and the further document of 13 May 2026 (schedule A and A2) also record: an email of the Director dated 7 August 2023 acknowledging “a rostering error that was accidentally made ... with regards to night shifts”; that a leave application for 20 to 27 February 2024 was declined for want of an attached statutory declaration when “a review indicates that in fact, the attachments were present”, described as “human error”; and that a payroll instruction to correct my shifts was issued on 3 May 2024 and the correction submitted on 28 May 2024, with an email of 21 May 2024 in the interval.

From a clinical perspective, does a sequence of that kind — repeated administrative error affecting rostering, leave and pay, in a role for which no fatigue assessment applied — bear differently on the condition you have diagnosed than a single isolated event would? And does the persistence of such matters over a period without resolution bear differently again from the same matters resolved promptly? Please explain the mechanism, if any.

2.4 The three stressors identified in the capability checklist. The employee capability checklist of 3 July 2026 identifies symptom exacerbation on exposure to (i) **complaint handling**, (ii) **being held accountable and blamed for the failures of others**, and (iii) **unpredictable rostering**. Please state the clinical basis on which each of those three is identified as contributing to the condition, by reference to the material.

As to (ii), the assumed facts include: a rostering error acknowledged in writing as having been made by another person; a leave application declined for a missing attachment which a review found “in fact... present”; and a payroll correction which another person was instructed to make and which took 25 days. **Whether, and by what mechanism if any, dealing with the consequences of administrative errors of that kind can contribute to or exacerbate the condition you have diagnosed.**

MATTER 3 — WAS THE EMPLOYMENT A SIGNIFICANT CONTRIBUTING FACTOR?

This matter has three parts, and I ask you to take them in the order given — the other factors and their timing first, then the interval and the mechanism, and your conclusion last, so that the conclusion rests on the two answers

before it. The questions on capacity, restrictions and adjustments follow at 3.4 to 3.8.

3.1 Other factors, placed in time. Please identify any non-employment factors relevant to (i) onset at or about **18 June 2024** and (ii) the subsequent course. Your report of 13 February 2025 referred to multiple life stressors including relationship breakdown, job loss and bereavement.

Your clinical relationship with me began on 24 October 2024, and my partner attended that consultation with me. Each of the matters above arose while I was under your care and is recorded in your own file.

In placing matters in time, the dates in the table at Part B may assist. The claim was rejected on 13 September 2024; I applied for review on 16 September 2024; the review decision issued on 24 October 2024, the same day our clinical relationship began; and the **appeal was filed on 26 November 2024.**

I ask you:

(a) to place each factor in time **from your own records rather than by assumption**, and to state for each **whether it arose before or after 18 June 2024**;

(b) to state, for any factor arising after that date, **whether it is capable of explaining the onset**; and to state whether, **as at the date of your diagnosis**, my relationship was a protective factor or an adverse one; and

(c) for each factor, whether in your opinion it was **independent of the employment matters, or a consequence of them** — noting that your report of 13 February 2025 records that my pay was “withheld or delayed for up to five months at a time, leading to significant financial stress and strain on his relationship”.

3.2 The interval, and the mechanism. The rostering and administrative matters at 2.2 and 2.3 occurred between August 2023 and May 2024, and the date of onset in this matter is **18 June 2024**. Is that interval consistent with, or inconsistent with, your formulation — and if consistent, **by what mechanism**? Separately, what significance, if any, attaches to the interval between onset and first presentation, the first workers' compensation medical certificate in the List of Documents being dated 1 July 2024? Please take into account your answer at **1.3(d)** as to the renewal of psychiatric referral on 16 May 2024.

3.3 Causation — your conclusion. On the assumed facts, and having regard to your answers at 3.1 and 3.2, whether my employment was **a significant contributing factor** to the injury with onset on 18 June 2024. Please give your clinical reasoning, and distinguish between the cause of onset and any factors bearing only on the subsequent course. **If your answer is yes, please identify which of the conditions or matters at 2.1 to 2.4 you consider contributed, and their relative weight** — so that the opinion identifies a mechanism rather than the employment at large. I do not ask you to find that employment was the only cause, or the main one — only whether, on the material, it was a significant contributing factor.

Capacity, restrictions and adjustments — the matters my employer has asked about. The employee capability checklist of 3 July 2026 records that **usual switchboard operational duties remain suitable**, subject to the restrictions it sets out; I ask only that your opinion be your own.

3.4 Prognosis and current capacity. Your prognosis; and, distinguishing (a) my capacity to perform the substantive role **with reasonable adjustments** from (b) any current incapacity. If there is current incapacity, its cause — the condition itself, the consequences of my having been away from the workplace since 3 July 2026 (a matter of my account, not of Attachments 1 to 3 or 5), or other factors.

The functional record is already in Attachment 4 — the employee capability checklist of 3 July 2026 sets out the pattern of work actually performed and the restrictions then in place. **You may take that record as established and need not reconstruct it.** What I ask for at 3.5 and 3.6 is your own opinion.

3.5 The adjustments, in functional terms. The adjustments you consider clinically necessary, the clinical basis for each, and the anticipated duration and review date — addressing, among anything else you consider relevant: predictability of rostering; adequate recovery between shifts; sustained concurrent emergency-code load; hours of work; and reporting arrangements. Please also address, in functional terms:

(a) any tasks, situations or environments likely to **exacerbate** the condition;

(b) if you consider a restriction on **complaint handling** clinically necessary, what activities it encompasses — receiving, documenting, redirecting or resolving complaints, or all complaint-related interaction. **My employer records that the established process for client complaints received by Switchboard employees is for immediate escalation to, and management by, the Client Liaison Officer and/or the Manager, Switchboard Services.** Please frame the restriction by reference to that process; and

(c) your report of 13 February 2025 records that my **working memory** is affected under stress — what that means for performance of the duties at Attachment 3, and the circumstances likely to give rise to it.

3.6 The requirements of the position. Attachment 3 describes the duties of the position and states that it is a continuous shift working role. Please address, in order:

- (a) whether the adjustments at 3.5 are **clinically necessary at present** — that is, whether I am presently able to perform the duties without them. **If your answer is that I am not, please identify which duties and why**, so that the answer is not read as a general incapacity for the role;
- (b) whether, **with those adjustments in place**, I am able to perform the duties described at Attachment 3;
- (c) whether adjustments of that kind — predictability of rostering, minimum recovery intervals between shifts, limits on sustained concurrent emergency-code load, and hours of work — are **of a kind ordinarily provided by large employers, and in particular by health services operating 24-hour services**; and
- (d) whether the adjustments are **permanent, or temporary with a review date**; and
- (e) Attachment 5 records changes to my working hours approved by my employer during 2026 — **56 hours per fortnight from 1 March, and 40 hours per fortnight from 25 May**, against a full-time fortnight of 76 hours, with the shift arrangement recorded on each form as “Continuous Shift Worker”. In your opinion, does a reduction of that kind indicate **deterioration in the condition, or the successful management of a stable one**?

I note that the employee capability checklist of 3 July 2026 at Attachment 4 records: “This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.” Please take that record into account in answering (b) and (c).

3.7 Directions and workplace discussions. My employer has asked whether I am able to follow a **reasonable and lawful direction** issued by my supervisor, and to **participate in discussions in relation to my workplace performance and/or conduct**. Please state whether there is any **clinical impediment** to either. If any adjustment would be clinically indicated for discussions of that kind — for example notice, a written agenda, the presence of a support person, or duration — please specify it and its clinical basis.

3.8 Foreseeable risk. Whether exposure to the conditions described at Attachments 2 and 3, **without** the adjustments at 3.5 and in the absence of a fatigue risk assessment, presents a foreseeable risk to my health or safety; and if so, what controls you consider medically necessary to manage it.

PART D — WHAT I DO NOT ASK, AND THE FORM OF THE REPORT

I do not ask you to express a view on whether any management action was reasonable; on whether my employer is able to accommodate the adjustments you recommend; or on my capacity to work under any particular reporting line or with any particular person. **Those are workplace questions, not medical ones. If you are asked any of them, I ask that you say so rather than answer.**

As to form, I ask that the report:

- (a) answer under the numbering used in this letter, so that each question and its answer can be identified — and where a question is not answered, say so under its number and why;
- (b) state your qualifications and specialty, and the dates and duration of each consultation on which your opinion is based;
- (c) reason from the assumed facts to your conclusions rather than assert conclusions, and state where the material is insufficient for you to express an opinion rather than qualifying an opinion you would not otherwise give;
- (d) state expressly which of the sources at Part B your opinion as to causation rests on, and whether that opinion depends on my account of the workplace matters;
- (e) state whether anything in the clinical records provided is inconsistent with, or qualifies, the opinions you express — and if so, how you have taken it into account, so that the report addresses the whole of the material rather than part of it; and
- (f) confirm that the opinions expressed are your own; and
- (g) be prepared as a standalone report, not as a letter to me. Please style it “Psychiatric report — in answer to the request for medical information made by Metro South Hospital and Health Service, 31 July 2026”, identify me only as the subject of the report, and address it to no one — **so that it reads the same in every hand it reaches**.

As to who will receive the report. It will be provided to both parties to my matter, and to no one else:

(i) to the **Workers' Compensation Regulator**, by way of continuing disclosure in the Commission proceeding — the same proceeding in which the Form 29 requested by Ms Matheson was served on your practice; and

(ii) to **Metro South Hospital and Health Service**, in answer to its request for medical information of 31 July 2026.

Both will receive the identical report, on the same day, together with this letter of instruction and the schedule of assumed facts — and each will be told that the other holds it. There will be no separate or edited version.

As to cost and instruction: the cost of the report is being met by Metro South Hospital and Health Service, which has requested medical information about my capacity for work. **The report is nonetheless instructed by me, and it is to be provided to me on completion.** The funding, and the identity of the party meeting it, bear in no way on the opinion you express.

APPENDIX — THE EMPLOYER'S REQUEST OF 31 JULY 2026, AND WHERE EACH QUESTION IS ANSWERED

This is the request the report answers. It is the nine-question schedule signed by **Mr Scott Hughes** on 31 July 2026 (Attachment 7), set out below in the employer's own terms, with the question in this letter that answers each. Where a question is not answered, the reason is that it goes to workplace arrangements rather than to medical capacity — I ask only that you say so.

Their question	Answered at
1(a) When was Mr Shepherd first diagnosed with MDD?	1.1
1(b) What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition?	2.1 · 2.2 · 2.3 · 2.4 and 3.3
1(c) Whether these workplace stressors are based solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you.	Part B and Part D(d)
1(d) Whether exposure to these workplace stressors presents a foreseeable risk to Mr Shepherd's health or safety if he were to return to work, and if so what specific workplace controls or adjustments you consider medically necessary to manage that risk.	3.8
2 Whether Mr Shepherd is able to follow a reasonable and lawful direction issued by his supervisor; and to participate in discussions in relation to his workplace performance and/or conduct.	3.7
3 Whether Mr Shepherd is medically fit to return to his substantive role under the existing reporting arrangements, including working with and reporting to his current line manager.	Not asked. A question about workplace arrangements — please say so rather than answer
4 If not, specify any restrictions or adjustments you recommend, the clinical basis in functional terms, and the anticipated duration and review date.	3.5
5 What specific activities are encompassed within “complaint handling” — receiving, documenting, redirecting, resolving, or all complaint-related interactions.	3.5(b)
6 Is Mr Shepherd able to fulfil the full inherent requirements of his role without restrictions or modifications to duties?	3.6(a)–(d)
7 What does “working memory is affected under stress” mean in functional terms — how it may affect capacity to perform the substantive duties under normal workplace demands, and the circumstances likely to cause the difficulty.	3.5(c)
8 Are there specific tasks, situations or environments that may exacerbate Mr Shepherd's condition or symptoms?	3.5(a)
9 If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?	Not asked. A question about workplace arrangements — please say so rather than answer

Schedule of assumed facts

WC/2024/227 · Shepherd · accompanying the letter of instruction · 17 August 2026

This schedule identifies, by source and paragraph, the facts referred to in Part B of the letter of instruction — **the facts you are asked to assume. The documents themselves accompany it, complete and unmarked.** Where a passage is quoted, the quotation is exact. **Section B records the matters the Workers' Compensation Regulator does *not* accept, so that the position is complete.**

A · ACCEPTED BY THE WORKERS' COMPENSATION REGULATOR — RESPONSE SIGNED BY MS RENEE MATHESON, SENIOR APPEALS OFFICER, 18 FEBRUARY 2026

Item numbers are those of the list to which that document responds; the cross-references have been checked against it. These are facts the Regulator accepts.

Item	Fact accepted
1	On 17–18 March 2024 the appellant was rostered to finish at 23:00 and commence his next shift at 06:00, a break of only 7 hours.
3	The employer's fatigue risk management policy and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement.
5	Ms Tammy Reese stated in an email of 7 August 2023 : <i>"I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts."</i>
8	Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety. It adds that <i>"there was a procedure in place for this to occur."</i>
20	On 24 December 2024 the Ethical Standards Unit determined that the appellant's complaint regarding Ms Taylor and Ms Reese constituted a Public Interest Disclosure.
33	The clinical records contain a consultation record dated 16 November 2023 authored by Dr Priyal De Silva Nanayakkara. (<i>Accepted as being in the record; its accuracy is not accepted — see B.</i>)
38	The report of Dr Ravikumar Krishnaiah states that the appellant was suffering from Major Depressive Disorder, diagnosed 13 February 2025. (<i>Accepted as what the report says; its accuracy is not accepted.</i>)
40	On 3 May 2024 Payroll Officer Elaine Grant emailed the line manager instructing her to <i>"submit an AVAC to correct these shifts"</i> for the appellant.
41	The AVAC was submitted on 28 May 2024. It adds that on 21 May the line manager had said she was waiting on payroll confirmation.
46	On 21 May 2024 at 12:33pm the line manager emailed the appellant: <i>"I am waiting payroll confirmation... as soon as I do get that confirmation, I will submit an AVAC."</i>

A2 · FROM A FURTHER DOCUMENT OF THE WORKERS' COMPENSATION REGULATOR, 13 MAY 2026

These are its own positive statements in that document.

Item	What it records
14(e) 14(f)	On the special pandemic leave application (leave of 20–27 February 2024), which was declined for want of an attached statutory declaration: <i>"says that a review indicates that in fact, the attachments were present on the appellant's submission"</i> · <i>"says this was a matter of human error by Ms Taylor on the background of high work demands surrounding the management of staff with COVID-19".</i>
22(a)	On the rostering of 17–18 March 2024: <i>"says the shift was separated by only a 7-hour break (rather than an 8-hour break) says that this was a result of human error and not intentional or repeated".</i>
22(c)	<i>"says the appellant took leave on 19 March 2024", and says this was paid leave.</i>

B · NOT ACCEPTED — AND WHAT THE REGULATOR SAYS INSTEAD

Recorded so that no fact in section A is read as broader than it is.

Item	Its position
4	Not accepted that there was no written agreement: <i>"in June 2020, the Appellant signed an agreement allowing an 8 hour break between shifts."</i> On that case the applicable minimum was 8 hours, and the break given was 7.

Item	Its position
34	Accepts the 16 November 2023 entry (" <i>No psychological illness such as depression/ psychosis</i> ") is in the record, but does not accept its accuracy.
35	Not accepted. It asserts a past medical history of anxiety and ADHD from 26 October 2022.
47	Not accepted that the line manager had simply held the instruction: it says " <i>Ms Taylor needed confirmation from payroll and then she needed to be satisfied that the AVAC was accurate.</i> " (The two emails at items 40 and 46 are accepted.)

C · FROM THE LETTER OF THE CHIEF EXECUTIVE, METRO SOUTH HEALTH, 5 JUNE 2026

Item numbers are those of the letter. Read from source.

Item	What the letter records
1–2	Pre-upgrade SPOK records were not retained. "However, a spreadsheet of recorded MET calls is available for the period 17-18 March 2024."
3(a)	<i>"All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant."</i>
3(c)	<i>"there have been no 'consequential' changes to operating procedures over the period requested."</i>
4	The requested fatigue risk assessment records do not exist. <i>"Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants. The Logan Hospital Switchboard staff are non-clinical staff..."</i>
5	<i>"The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard."</i>
7	The requested fatigue risk management register entries do not exist.
15	Enclosed: leave takings report for the Applicant, 19 March 2024.

D · FROM THE ROLE DESCRIPTION — ADMINISTRATION OFFICER, SWITCHBOARD SERVICES (AO3)

Quoted from the employer's own document. Read from source.

Under	The position requires
Purpose	<i>"The occupant of this position is required to work continuous shift work over the full 24-hour period, 7 days a week."</i>
Key responsibilities	<i>"Participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes."</i>
	<i>"Maintain call queues to minimum at all times." · "The ability to multitask and operate under pressure, particularly where high volume call traffic is concerned."</i>
	<i>"Maintain discretion and exercise judgement where necessary to resolve problems within the scope of your role; in situations where precedence have not been set and procedures not defined."</i>
	<i>"Ability to work effectively as an individual with limited supervision..."</i>
	<i>"Collate information and maintain Omnivista database and SharePoint to ensure information held within Switchboard Services is accurate and appropriate."</i>
Mandatory requirements	<i>"The position is a continuous shift working role. You must be able to work a roster which covers multiple shifts over a 24/7 period."</i>

E · FROM THE EMPLOYEE CAPABILITY CHECKLIST, 3 JULY 2026

Read from source. Quotations exact.

Question	What the checklist records
1	<i>“Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024... Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).”</i>
2	<i>“Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.”</i>
3	<i>Six 8-hour shifts per fortnight (of ten in a full-time fortnight) — no more than two weekday day/afternoon, two weekend day, two night. ■ “This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration... Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.”</i>
4	<i>“(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination through the Injury Management team; (d) access to EAP; (e) review in approximately 8 weeks.”</i>
Meds	<i>Fluoxetine 60mg mane · lisdexamfetamine 30mg mane for cognitive symptoms and focus · quetiapine 25mg nocte ceased. “maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication.”</i>

F · FROM THE APPROVED CHANGES TO WORKING HOURS, 2026 — ATTACHMENT 5

Fields copied from the employer's own forms. Full-time fortnight for the position: 76 hours.

Delegate approval	Effective period	Hours / fortnight	Shift arrangements
27.02.2026	01.03.2026 – 15.03.2026	56.00	Continuous Shift Worker
17.04.2026	16.03.2026 – 26.04.2026	56.00	Continuous Shift Worker
09.06.2026	25.05.2026 – 28.06.2026	40.00	Continuous Shift Worker

Each form records the movement task as *“Change to Working Conditions (Temporary)”* and the position as **AO Switchboard, AO3, Level 04**.

G · Written on the Form 29 — Attachment 6

Item	Written on the Form 29
Requesting party	Workers' Compensation Regulator
Signed by	Ms Renee Matheson, Senior Appeals Officer — 4 July 2025
Sealed	Industrial Registry, 4 July 2025
Nominated party	The Practice Manager, Mind and Memory Service
Matters in issue	1. Did Mr Shepherd sustain a personal injury 2. Did the personal injury arise out of or in the course of Mr Shepherd's employment 3. Was Mr Shepherd's employment a significant contributing factor to the injury

H · Written on the movement forms and the 31 July 2026 request

H1 · Attachment 5

Delegate approval	Effective period	Hours / fortnight	Delegate
27.02.2026	01.03.2026 – 15.03.2026	56.00 · Continuous Shift Worker	Scott Hughes
17.04.2026	16.03.2026 – 26.04.2026	56.00 · Continuous Shift Worker	Scott Hughes
09.06.2026	25.05.2026 – 28.06.2026	40.00 · Continuous Shift Worker	Scott Hughes

H2 · Attachment 7

Item	Written on the request
Signed by	Mr Scott Hughes, Director, Corporate Services, Logan Beaudesert Health Service
Date	31 July 2026
Nine questions	Written on Attachment 7. Mapped in the appendix to the letter of instruction.
Not asked	Questions 3 and 9.

I · Written on the letter of Ms Noelle Cridland — Attachment 2

Item	Written on the letter
Signed by	Ms Noelle Cridland, Chief Executive, Metro South Hospital and Health Service
Date	5 June 2026
Reference	K-LM26/729
Addressed to	Commissioner Dwyer, Queensland Industrial Relations Commission
Passages used	Section C of this schedule. Those passages are written on Attachment 2.

Form 24 – Notice to admit facts

Industrial Relations Act 2016, section 989
Industrial Relations (Tribunals) Rules 2011, rules 41, 49, 108 and 113

seal

Applicant/Appellant:	Cory Lea Shepherd
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V

Respondent:	
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PLEASE NOTE:
If there are more than two parties to this application, please complete a **Form 1 – Parties list** and file it with this form.

To:	
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Take notice that the ○ applicant ○ appellant ● respondent in this proceeding proposes to prove the facts specified below, and if you do not within 14 days serve a notice on the ○ applicant ○ appellant ● respondent disputing the facts you are taken to admit, for this proceeding only, the facts specified in this notice.

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
1	On 17–18 March 2024 , the Appellant was rostered to finish work at 23:00 and commence his next shift at 06:00, resulting in a break of only 7 hours (Exhibit C2) .	
2	In the Review Decision dated 24 October 2024 (Exhibit B1 , p. 27), the Review Officer found that rostering the Appellant with a 7-hour break " <i>amounted to unreasonable management action</i> ".	
3	The Employer's "Fatigue Risk Management Policy" (Exhibit F1) and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement.	
4	No written agreement existed for the Appellant to work an "Eight-hour break" shift pattern in March 2024.	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
5	Ms. Tammy Reese admitted in an email dated 7 August 2023 : <i>"I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts"</i> (Exhibit E1 , p. 20).	
6	On 6 June 2023 , Line Manager Ms. Chloe Taylor removed a page containing the Appellant's handwritten entry from the workplace Communication Book (Exhibit I2).	
7	The entry removed by Ms. Taylor contained operational information regarding the necessity of verifying and updating doctors' contact details.	
8	Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety.	
9	By removing this entry without consultation, Ms. Taylor prevented the communication of safety-critical information to other staff members.	
10	In an email to Ms. Reese dated 6 June 2023 at 4:05 PM (Exhibit E1 , p. 10), Ms. Taylor referred to the Communication Book—a government record—as a <i>"burn book"</i> .	
11	The term "Burn Book" is a specific pop-culture reference to the film <i>Mean Girls</i> , describing a book used to spread rumours, insults, and social gossip.	
12	Ms. Taylor's characterisation of a safety-related record as a "burn book" indicates she viewed the workplace through a social clique lens rather than an objective professional assessment.	
13	Ms. Carolyn Jeffrey (witness) provided a statement (Exhibit I2) corroborating that Ms. Taylor <i>"rips the page out"</i> and confirmed the action was <i>"humiliating for Cory"</i> .	
14	In the email to Ms. Reese dated 6 June 2023 (Exhibit E1 , p. 10), Ms. Taylor explicitly admitted: <i>"I did raise my voice and asked him to please stop talking over the top of me."</i>	
15	Ms. Jeffrey's statement (Exhibit I2) corroborates this event, stating Ms. Taylor <i>"yelled at Cory over something that did not warrant her reaction"</i> .	
16	Raising one's voice at a subordinate in an open office environment constitutes unprofessional conduct and is not "reasonable management action".	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
17	The "Union Encouragement Policy" (QH-POL-248) (Exhibit F2) requires managers to take a "positive, supportive role" to facilitate union membership and delegate elections.	
18	The Appellant notified management of his intent to become a Union Delegate on 11 August 2023 (Exhibit F3).	
19	Management failed to facilitate a vote or provide necessary delegate information to the Appellant for a period exceeding 9 months after his initial request.	
20	On 24 December 2024 , the Ethical Standards Unit (ESU) determined that the Appellant's complaint regarding Ms. Taylor and Ms. Reese constituted a Public Interest Disclosure (PID) (Exhibit E3).	
21	On 15 May 2024 , Director Tammy Reese sent an email directing the Appellant to "retract" an email he had sent inquiring about office hours (Exhibit E5).	
22	On 9 May 2024 , Ms. Taylor sent a substantively similar email regarding another staff member's hours, for which she was not directed to issue a retraction (Exhibit E5).	
23	The Respondent obtained the Appellant's full medical history (Exhibit A5) directly from "Our Medical Ashmore" on or before 8 July 2025 .	
24	The copy of Exhibit A5 filed by the Respondent bears the system-generated footer: "Printed on 8th July 2025".	
25	The Respondent did not serve a Form 29 Notice of Non-Party Disclosure on the Appellant prior to obtaining these records (breach of Rule 64E).	
26-31	<i>Items 26 to 31 are not included in this copy: they concern matters with no bearing on the medical questions.</i>	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
32	On 14 July 2025 , the Respondent's representative (Renee Matheson) emailed the Appellant stating she had " <i>received last week</i> " documents but did not disclose possession of Exhibit A5 (Exhibit H1) .	
33	Exhibit A5 (p. 52) contains a consultation record dated 16 November 2023 authored by Dr. Priyal De Silva Nanayakkara.	
34	The entry dated 16 November 2023 explicitly states: " <i>No psychological illness such as depression/ psychosis.</i> "	
35	The Respondent is not in possession of any contemporaneous medical evidence prior to 2024 that contradicts the finding in Item 34.	
36	The Work Capacity Certificate issued by Dr. Peter Hawes dated 1 July 2024 (Exhibit A1) identifies the mechanism of injury as " <i>ongoing breaking of workplace rules by bosses, victimizing him</i> ".	
37	In the Review Decision (Exhibit B1 , p. 16), the Regulator stated: " <i>I am satisfied your employment was a significant contributing factor to the psychological injury</i> ".	
38	Dr. Ravikumar Krishnaiah (Psychiatrist) diagnosed the Appellant with Major Depressive Disorder on 13 February 2025 (Exhibit A4) .	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
39	Dr. Krishnaiah opined that premature exposure to the workplace is likely to result in <i>"significant deterioration in my mental health"</i> (Exhibit A4 , p. 5).	
40	On 3 May 2024 , Payroll Officer Elaine Grant sent an email to Ms. Taylor instructing her to <i>"submit an AVAC to correct these shifts"</i> for the Appellant (Exhibit E2).	
41	Ms. Taylor did not submit the required AVAC until 28 May 2024 (Exhibit D2), a delay of 25 days despite the explicit written instruction from Payroll.	
42	Exhibit D1 (Employee Payslip Comparison) demonstrates a total gross pay for the comparator employee of \$18,169.14 over the sample period.	
43	Exhibit D1 demonstrates a total gross pay for the Appellant of \$12,757.68 over the same period.	
44	This represents a gross pay disparity of \$5,411.46 (approximately 42%) to the Appellant's detriment.	
45	This wage gap was caused primarily by the differential allocation of penalty-rate shifts (weekends and public holidays) to the comparator employee rather than the Appellant.	
46	On 21 May 2024 at 12:33 PM , Ms. Taylor emailed the Appellant stating: <i>"I am waiting payroll confirmation... as soon as I do get that confirmation, I will submit an AVAC"</i> (Exhibit E2 , p. 236).	
47	At the time of sending the email described in Item 46, Ms. Taylor had already been in possession of written instructions from Payroll Officer Elaine Grant since 3 May 2024 (Exhibit E2 , p. 237).	
48	Ms. Taylor's statement on 21 May 2024 that she was "waiting payroll confirmation" was false or misleading, as the instruction had been provided to her 18 days earlier.	
49	This conduct demonstrates a lack of diligence and transparency in managing the Appellant's financial entitlements.	
50	In the Review Decision (Exhibit B1), the Regulator attributed the payroll errors to "human error," but failed to address the misleading statement made by Ms. Taylor on 21 May 2024.	

Signature:	
Print name:	
Title of office held:	

Date:

____ / ____ / _____

Respondent's response to the appellant's notice to admit facts

Industrial Relations Act 2016, section 989
Industrial Relations (Tribunals) Rules 2011, rules 41 and 49

seal

Appellant:	Cory Shepherd
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v

Respondent:	Workers' Compensation Regulator
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PLEASE NOTE: If there are more than two parties to this application, please complete a **Form 1 – Parties list** and file it with this form.

To:	Cory Shepherd
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Take notice that the Respondent in this proceeding responds to the Appellant's notice to admit facts as follows:

	Respondent's Response
1.	With respect to paragraph 1, admits the facts contained therein.
2.	With respect to paragraph 2, does not admit the fact contained therein because the appeal is a hearing de novo and it is for the Commission to determine if this fact amounted to unreasonable management action.
3.	With respect to paragraph 3, admits the facts contained therein.
4.	With respect to paragraph 4, denies the fact because in June 2020, the Appellant signed an agreement allowing an 8 hour break between shifts. This fact also appears at paragraph 34 of the Appellant's statement of facts and contentions, which have been filed in the Commission.
5.	With respect to paragraph 5, admits the facts contained therein.
6.	With respect to paragraph 6, admits that Ms Taylor removed a page from the workplace communications book but does not admit that the page contained the appellant's handwriting because the respondent does not have a copy of the page.
7.	With respect to paragraph 7, does not admit the facts contained therein because the Respondent does not have a copy of the document referred to and is unaware of its contents.
8.	With respect to paragraph 8, admits the facts contained therein and says that there was a procedure in place for this to occur.
9.	With respect to paragraph 9, denies as untrue the fact, because there was already a procedure in place.
10.	With respect to paragraph 10, does not admit the fact that the email dated 6 June 2023 at 4.05 pm Ms Taylor refers to the Communication book as a "burn book" because the email states that Ms Taylor

	Respondent's Response
	felt like the Communications Book was being used as a “burn book” and not being used a professional tool. Further the Respondent says the email as a whole, needs to be read to determine the meaning of the phrase and context. Further, the Respondent denies as untrue that the Communications book is a government record
11.	With respect to paragraph 11, does not admit the fact, because the Respondent it unsure of the relationship to the film Mean Girls.
12.	With respect to paragraph 12, denies as untrue because Ms Taylor identified the Communications Book as “a professional tool” that switchboard can use to communicate.
13.	With respect to paragraph 13, the respondent has viewed an email from Ms Jeffrey dated 18 July 2024 where she states the words “rips the page out” and “humiliating for Cory”
14.	With respect to paragraph 14, admits the facts contained therein.
15.	With respect to paragraph 15 the Respondent does not admit this fact because the Respondent has viewed an email from Ms Jeffrey dated 18 July 2024 where she states the words “ <i>yelled at cory over something that did not warrant her reaction</i> ” but confirms that she did not witness the event and was told about it by the Appellant.
16.	With respect to paragraph 16, denies as a fact, and says that the characterisation of any management action is a matter for determination by the Commission.
17.	With respect to paragraph 17, admits the Policy says words to that effect.
18.	With respect to paragraph 18, admits that the Appellant sent a text to Ms Taylor at 5.03pm that he was “just putting his hand up” but says that this was not a formal notification.
19.	With respect to paragraph 19, denies as untrue the facts, because: • the Appellant did not request assistance; • Ms Taylor did provide advice and support to the Appellant; • the Appellant was provided with delegate information.
20.	With respect to paragraph 20, admits the facts contained therein.
21.	With respect to paragraph 21, admits the facts contained therein but says there was no correlation between the two events. The Respondent says further that Ms Reese also emailed the Appellant about retracting the email on 21 May 2024 and all of the emails need to be read as a whole to determine the meaning and context
22.	With respect to paragraph 22, denies as untrue the facts, because the circumstances were different and did not require Ms Taylor to retract her email.
23.	With respect to paragraph 23, denies as untrue the fact that the Respondent obtained the Appellant’s full medical history and says that the Respondent obtained records in a date range in line with Form 29 - Notice of non-party disclosure, signed by a Senior Registry Officer on 4 July 2025, and the Respondent received the medical records some time before 16 July 2025.
24.	With respect to Paragraph 24 does not admit the facts, as there is no footer on the medical records but does admit the records state on page one that the records were printed on 8 July 2025. The Respondent is unable to verify the source of the date but admits that the Form 29 - Notice of non-party disclosure, was signed by a Senior Registry Officer on 4 July 2025 and that the medical records were received by the Respondent at some time before 16 July 2025.
25.	With respect to paragraph 25, admit the fact and says that the Respondent did not serve a Form 29 – Notice of Non-Party Disclosure on the appellant prior to obtaining the records and says there was no requirement to do so pursuant to section 64D <i>Industrial Relations (Tribunals) Rules</i> 2011. The Respondent denies as untrue that the Respondent breached Rule 64E.

	Respondent's Response
26.	With respect to paragraphs 26 to 29 denies the fact that details contained in the records are not relevant because, read as a whole, the medical records are relevant in that they tend to prove or disprove an allegation in the proceeding.
27.	With respect to paragraph 30, admits the fact that the Respondent served the medical records on the Appellant's solicitors on 22 July 2025 and says further the Respondent was not notified that the Appellant's solicitors no longer acted for the Appellant until 29 July 2025.
28.	With respect to paragraph 31, does not admit the facts because these are matters within the Appellant's knowledge.
29.	With respect to paragraph 32, admits that the Respondent sent an email directly to the Appellant on 14 July 2025 and says that the email addressed to the Appellant on that date was in error and that the email should have been sent to the Appellant's solicitors, because he was legally represented. The Respondent states that the medical records were not in the possession of the Regulator at the time of sending the email on 14 July 2025.
30.	With respect to paragraph 33, admits the fact that there is an entry from Dr Nanayakkara in the records but does not admit the accuracy of the record.
31.	With respect to paragraph 34, admits the entry is listed in the record, but does not admit the accuracy of the entry.
32.	With respect to paragraph 35, denies untrue the fact because the records show a past medical history of Anxiety and ADHD from 26 October 2022 which is missing from Exhibit A5.
33.	With respect to paragraph 36, admits the fact Dr Peter Hawes provided a medical certificate stating that the mechanism was "ongoing breaking of workplace rules by bosses, victimizing him" but denies that as a fact and further denies the date of the certificate is 1 July 2024 because the certificate is signed 8 September 2024.
34.	With respect to paragraph 37, admits the contents of the Respondent's review unit decision but says that this fact is not relevant because the appeal is a hearing de novo and it is for the Commission to determine. action
35.	With respect to paragraph 38, admits that the report does say that the Appellant was suffering from Major Depressive Disorder, but does not admit the accuracy of the report.
36.	With respect to paragraph 39, admits that the report does say: <i>"Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health..."</i> , but does not admit the accuracy of the report.
37.	With respect to paragraph 40, admits the facts contained therein.
38.	With respect to paragraph 41, admits the fact that Ms Taylor submitted the AVAC on 28 May 2024 but also says that on 21 May 2024 Ms Taylor emailed the appellant stating she was waiting for payroll confirmation on a few issues and would submit an AVAC for the next pay run as soon as she received it.
39.	With respect to paragraphs 42 to 45, denies untrue the fact, because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable.
40.	With respect to paragraph 46, admits the facts contained therein.

	Respondent's Response
41.	With respect to paragraphs 47, denies untrue the fact, because Ms Taylor needed confirmation from payroll and then she needed to be satisfied that the AVAC was accurate.
42.	With respect to paragraph 48, denies as untrue the fact, because the statement was not false or misleading.
43.	With respect to paragraph 49, denies as untrue the fact, because Ms Taylor's conduct does not lead to a conclusion that there was a lack of due diligence or transparency in managing the Appellant's financial entitlements.
44.	With respect to paragraph 50, admits the contents of the Respondent's Review Unit decision and says that this fact is not relevant because the appeal is a hearing de novo and it is for the Commission to determine. Does not admit that Ms Taylor made a misleading statement

Name:	Workers' Compensation Regulator		
Name of contact person:	Ms Renee Matheson		
Postal address:	150 Mary Street		
	Brisbane		4001
Phone number:	07 3842 2792	Fax number:	07 3811 6443
Mobile number:			
Email address:	Renee.Matheson@oir.qld.gov.au		
Signature:			
Date:	18/02/2026		

Attachment 2 — letter of Ms Noelle Cridland

WC/2024/227 · *Shepherd v Workers' Compensation Regulator*

Item	Written on the document
Document	Letter of the Chief Executive, Metro South Hospital and Health Service
Signed by	Ms Noelle Cridland, Chief Executive
Date	5 June 2026
Reference	K-LM26/729
Addressed to	Commissioner Dwyer, Queensland Industrial Relations Commission



Office of the Chief Executive

Enquiries to: Myla Ruttan
Principal Lawyer
Metro South Legal Services
Telephone: 07 3069 4755

Email: metrosouthlegal@health.qld.gov.au
Our Ref: K-LM26/729
Date: 5 June 2026

**Metro South
Health**

Commissioner Dwyer
Queensland Industrial Relations Commission
Level 21 66 Eagle Street Brisbane
Qld, 4000
By email: qirc.registry@qirc.qld.gov.au

Dear Commissioner Dwyer

WC/2024/227 – Cory Lea Shepherd v Workers' Compensation Regulator

I refer to the Form 29 – Notice of non-party disclosure (the **Notice**) regarding the above matter. Mr Shepherd is a current employee of Metro South Hospital and Health Service (**Metro South Health**) as Switchboard Operator at Logan Hospital.

The Notice compels Metro South Health to produce various documents that Mr Shepherd claims to require for his Workers' Compensation Appeal.

Metro South Health has complied with some of the Items requested in the Notice as outlined below. In accordance with Rule 64E(4) of the *Industrial Relations (Tribunals) Rules 2011*, our objections to production/compliance with the Notice are also outlined below.

At the outset, it should be noted that Metro South Health is not in receipt of the Applicant's Amended Statement of Facts and Contentions (Form 9A). Throughout the Notice, the Applicant has expressed various opinions and views about the relevance of the documents to the allegations and stressors the subject of the appeal. While Metro South Health's substantiative grounds of objection to compliance with the Notice are outlined below, our ability to determine whether the requested documents are 'relevant' to the proceeding (and therefore give rise to an objection under Rule 64E4(b)) has, to some extent been limited. We submit this ought to be taken into consideration by the Commissioner.

Items 1 and 2 - SPOK emergency paging export and Aggregate SPOK activation counts

Metro South Health cannot comply with the request for Items 1 and 2. The requested records do not exist. The system was upgraded in SPOK, and records predating that upgrade were not retained. However, a spreadsheet of recorded MET calls is available for the period 17-18 March 2024.

Item 3(a) - Complaints received - server side exchange filter

Metro South Health objects to the production of Item 3(a). The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)).

Better together

All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant.

Metro South Health's core function is to provide health services to the public. Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden. Particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024). In circumstances where the Matter in Issue as pleaded in the Notice relates to "consecutive shifts of 17 to 18 March 2024", Metro South Health submits a search of the requested range and breadth will take an unreasonable amount of time and resourcing and detract from our core functions of providing health care to the community.

Item 3(b) - Management discussion of complaints

Metro South Health relies on the same grounds for objection as for Item 3(a) above.

Item 3(c) - Consequential changes to operating procedures

Metro South Health cannot comply with the request for Item 3(c) because there have been no 'consequential' changes to operating procedures over the period requested.

Item 4 - FRMS risk assessment record

Metro South Health cannot comply with the Notice in relation to Item 4. This is because the requested documents do not exist. Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants. The Logan Hospital Switchboard staff are non-clinical staff, and therefore there is no mandatory requirement for them to complete Fatigue Risk Management System training.

Item 5 - FRMS training and competency records

Metro South Health cannot comply with the request in relation to Item 5. The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard.

Item 6 – FRMS delegation and governance record

Metro South Health encloses the attached Role Description for Manager of Switchboard Services.

Item 7 - FRMS register entries and switchboard roster expectations

Metro South Health cannot comply with the Notice in relation to Item 7. This is because the requested documents do not exist. Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants.

Item 8 – Management System Log in records

Metro South Health objects to the production of Item 8. The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)).

Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden, particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024) and a search of the requested

range and breadth will take an unreasonable amount of time and resourcing and detract from our core functions of providing health care to the community. In circumstances where the Matter in Issue as pleaded in the Notice relates to “consecutive shifts of 17 to 18 March 2024”, Metro South Health submits this date range is a reasonable range to search.

Further, the reference to “*Management system login records - Active Directory / Entra ID / Citrix. Native electronic 2024 export of Windows Active Directory, Entra ID and Citrix session-login audit records*” is unreasonably broad (lacking particularity). Metro South Health uses various application platforms and it is not clear which system is being referred to.

Item 9 - Management building access

Metro South Health objects to the production of Item 9 on the basis that the request lacks relevance, particularity and could affect a person (Rule 64E(4)(b)(c) and (f)). The Switchboard office is collocated in an office area where there are other staff from various departments going through the office area. The switchboard office does not have its own access-swipe access.

The Applicant is a current employee and reports directly to the line manager. The request is unreasonably broad (lacks particularity) and production could have an effect on the line manager.

Item 10 - various

Metro South Health objects to the production of the documents requested in Item 10. The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)). Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden. Particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024).

In circumstances where the Matter in Issue as pleaded in the Notice relates to “consecutive shifts of 17 to 18 March 2024”, Metro South Health submits a search of the requested range and breadth will take an unreasonable expenditure of time and resourcing, and detract from our core functions of providing health care to the community.

Item 11 - Appellant's MyHR audit trail - COVID Special Pandemic Leave (Process Reference 15480560)

Metro South Health encloses the following documents in compliance with the request for Item 11

- MyHR report leave submission for period 1 February 2024 to 31 May 2024;
- Leave form evidence of pandemic leave for period 20 to 27 February 2024;
- Leave form for period 20 to 27 February 2024;
- AVAC history for pandemic leave for the period 20 to 27 February 2024.

Item 12 – Comparator MyHR audit trails

In compliance with the request for Item 12, Metro South Health encloses a complete audit trail of the requested pandemic leave for the period requested for Logan Hospital Switchboard.

Metro South Health otherwise objects to the production of other documents as outlined in Item 12.

This request would require accessing the relevant staff members' supporting evidence of their Special Pandemic Leave requests during the period 1 January 2024 – 30 June 2024. Examples of supporting evidence could be a photograph of a positive COVID test, medical certificate, lab results confirming COVID diagnosis, or letter from their child's school in the event of school closure.

This request is unreasonable in circumstances where the effect of production would have on the other staff members (i.e. breach of their right to their privacy and reputation) and that the proof of COVID test contains confidential and sensitive personal health information of the relevant staff members.

Item 13 – Line Manager authority framework – COVID Special Pandemic Leave processing

In compliance with the request for Item 13, we enclose:

- HR Delegation COVID-19 Pandemic Event – Paid Special Pandemic Leave signed 23 November 2022;
- Pandemic Directive 01/20 – Employment Arrangements in the event of a Health Pandemic
- Implementation Guideline – Directive 01/20 dated 14 April 2020.

Item 14 - Text message and electronic communication corpus - Appellant and Line Manager concerning COVID Special Pandemic Leave

The requested information in Item 14 is not in control or possession of Metro South Health. The line manager was not issued with a Queensland Health mobile device, and as such the SMS messages exchanged between the Applicant and the Line manager relating to the pandemic leave request was made using the line manager's personal mobile device. The Queensland Health retention policy does not apply to employee's personal mobile.

The Switchboard line manager was issued with Queensland Health mobile device in or around the third quarter of 2024.

Item 15 - Leave ledger - 19 March 2024

In compliance with the request for Item 15, we enclose:

- Leave takings report for the Applicant 19 March 2024.

Item 16 - AVAC payroll export - Appellant (PRN 15397775 and PRN 15605601).

In compliance with the request for Item 16, we enclose:

- Daily Staffing variance form for the period 5 to 18 February 2024.
- Daily Staffing variance form for the period 19 February 2024 to 3 March 2024.

Item 17 – Comparator payroll export

Metro South Health objects to the production of Item 17 on the basis of Rule 64E(4)(e). Disclosing the requested information would infringe upon the human rights to privacy and reputation of the team members within the switchboard team, because:

- The staff members' identity can be easily re-identified even if redacted.
- The Logan Hospital Switchboard team is comprised of a small team.
- Given that the Applicant is a current employee within the same team, re-identification of the other team member would create a real risk of harm to their reputation.

Item 18 - Communication Book and 15 November 2023 Business Case minutes

Metro South Health objects to the production of Item 18 on the grounds of irrelevance (Rule 64E(4)(b)). The Notice indicates that the destruction of the record had already been pleaded at Form 9A, Part B, para 2 Stressor I(b) (Form 24, Para 14). In those circumstances, it is difficult to identify the relevance of the present request, given that the issue appears to have already been conceded.

Item 19 - Union delegate correspondence - server-side Exchange server

Metro South Health objects to the production of Item 19 on the basis of Rule 64E(4)(a) and (f).

Metro South Health employed approximately 20,006 MOHRI occupied headcount during the period of the dates of the Notice. The requested documents require searching through these staff's email messages received from Together Queensland. Compliance with a request of this date range and email recipients would require approval to engage external resources and assistance from eHealth Queensland at the Department of Health, resulting in a significant administrative and operational burden. We consider that the date range ought to be narrowed to 17-18 March 2024, limited to emails received from Together Queensland by the Switchboard line manager and their Director, or by members of the Logan Hospital Human Resources team who were employed by and remained with Metro South Health.

Further, any disclosure of correspondence involving Together Queensland would also require consultation with or issuing a production notice to them (as an affected party), if they were included in the email communication.

Item 20- Operational HR reaction to PID 24-ESU-1130

The information requested in item 20 does not exist, as the reference cited in the Notice being "PID24-ESU-1130" commenced in November 2024, which falls outside the specified search period of 13 May 2024 to 31 May 2024.

In the event you have any questions or concerns about this correspondence, I have arranged for a Principal Lawyer from the Metro South Health Legal team to be available to assist. Please contact metrosouthlegal@health.qld.gov.au

Yours sincerely



Digitally signed by
Noelle Cridland
Date: 2026.06.05
14:54:08 +10'00'

Ms Noelle Cridland

Chief Executive

Metro South Hospital and Health Service

Enc. Documents relating to items 6, 11, 12, 13, 15 and 16 of the Notice



Administration Officer, Switchboard Services (RD)

Job Ad Reference:			
Status:	Administration Officer, Switchboard Services		
Unit/Department:	Switchboard, Corporate Services		
Location:	Logan Hospital, Metro South Hospital and Health Service		
Contact:	Chloe Taylor (07) 2891 9190	Salary range:	
Classification:	A03	Closing date:	
Online applications:			
Email applications:	Chloe.taylor3@health.qld.gov.au		

****Applications from third parties will not be accepted****

Purpose of the role

To provide a highly effective and efficient communication service to the public and staff of the Logan Hospital within Metro South Hospital and Health Service.

The occupant of this position is required to work continuous shift work over the full 24-hour period, 7 days a week

Your key responsibilities

- This position reports to the Switchboard Manager, Corporate Services, Logan Hospital.
- Attend courteously to all telephone enquiries and clients' requests, displaying tact, diplomacy and interpersonal skills of a high standard when dealing with the public and members of staff. Maintain call queues to minimum at all times.
- Collate information and maintain Omnivista database and SharePoint to ensure information held within Switchboard Services is accurate and appropriate.
- Provide a service to support hospital staff in the allocation, coordination, and fault repair of all pager units for the hospital. This includes allocating pagers to hospital staff and submitting paging units for repair, maintaining database and registers with accurate and current information.
- Participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes.
- Ability to communicate at all levels and effectively establish and foster networks throughout the hospital to maintain accurate and current records for effective switchboard operations.
- Maintain discretion and exercise judgement where necessary to resolve problems within the scope of your role; in situations where precedence have not been set and procedures not defined.
- Interact in a sensitive, timely, effective, and appropriate manner with members of the general public and service providers, maintaining strict confidentiality at all times.
- The ability to multitask and operate under pressure, particularly where high volume call traffic is concerned.
- Contribute to a cohesive team environment by sharing information, supporting and assisting colleagues in a proactive manner.
- Ability to work effectively as an individual with limited supervision to meet deadlines and establish work priorities, and work as a member of a multi-disciplinary team in a changing environment.
- Ensure patients and other staff are treated fairly and with mutual respect and understanding, regardless of cultural, religious, ethnic and linguistic backgrounds, gender, identity, ages or abilities.

- Follow defined service quality standards, occupational health and safety policies and procedures relating to the work being undertaken to ensure high quality, safe services and workplaces.

Mandatory qualifications, professional registration and other requirements

- The position is a continuous shift working role. You must be able to work a roster which covers multiple shifts over a 24/7 period.
- Intermediate / advanced computer skills.
- Recent experience in the operation of a switchboard is desirable but not essential.
- **Vaccine Preventable Disease (VPD):** It is a condition of employment for this role for the employee to be, and remain, vaccinated against the following vaccine preventable diseases during their employment ([Health Employment Directive No. 01/16](#)):
 - measles, mumps, rubella (MMR)
 - varicella (chicken pox)
 - pertussis (whooping cough)
 - hepatitis B
- It is a condition of employment for this role for the employee to be, and remain, vaccinated against COVID-19 ([Health Employment Directive No. 12/21](#) and [Queensland Health Human Resources Policy B70](#)).

Are you the right person for the job?

How we do things is as important as what we do, therefore within the context of the responsibilities described under 'Your key responsibilities,' the ideal applicant will be someone who can demonstrate the following:

Communicate Effectively	▪ Strong communication and negotiation skills to understand customers' needs. Demonstrate respect, courtesy and fairness when interacting with patients, clients and colleagues.
Team Focus	▪ Value diversity of thought and include everyone within the team. Actively support a culture of diversity, trust, support and inclusion.
Technical Skills	▪ Intermediate / advanced computer skills.
Change & Quality Focus	▪ Be open to new ideas and approaches and be adaptable in the face of change. Support and participate in change initiatives. Follow procedures and processes understanding implications within a health care setting.
Work Ethics	▪ Be honest, ethical and professional. Work within boundaries of role.

How to apply

Please provide the following information to the panel to assess your suitability:

- A short written response (maximum 1-2 pages, dot points acceptable) on how your experience, abilities, knowledge and personal qualities would enable you to achieve the key responsibilities and meet the skill requirements of the role.
- Your current CV or resume, including two referees. You must seek approval prior to nominating a person as a referee.
- Late applications cannot be submitted online. For a late application to be considered, please arrange approval and submission via the contact person.
- Applications will remain current for 12 months or for the duration of the vacancy. Future vacancies of a temporary, full time and part time nature may also be filled through this recruitment process.
- Only those persons eligible to work in Australia may be employed by Queensland Health. Prospective employees are required to provide proof of identity and documentary evidence of their [right to work in Australia](#).

We are Metro South Health

We are the major public healthcare provider for Brisbane's south side, Logan, Redlands and the Scenic Rim. We operate five major hospitals and a range of community, specialty and state-wide healthcare services. We are renowned for teaching and research excellence.

We are truly dedicated people who care about our community. We come to work to make a difference. We save lives, changes lives and make the world a better place. We overcome the many challenges that come our way by going above and beyond and achieve the best possible outcomes by working together. We are bold, innovative, collaborative, inspiring.

We are part of and proud of the community we serve. We are *better together*.

Our values

Our values of Integrity, Compassion, Accountability, Respect, Engagement and Excellence, shape our culture. Our values guide our day-to-day decision making and are fundamental to what we care about as a health service, how we behave, how we interact with each other and provide care to the many patients who come through our doors every day.



INTEGRITY

We are authentic, truthful, and transparent, and strive for equity for all.



COMPASSION

We care for one another and ourselves with empathy, kindness and support.



ACCOUNTABILITY

We are accountable for our decisions, actions and behaviour.



RESPECT

We foster an environment of safety, civility, and inclusion.



ENGAGEMENT

We are one team working together to achieve our best outcomes.



EXCELLENCE

We empower each other and inspire innovation to deliver excellence.

Diversity and inclusion

We are the most culturally diverse area of Queensland with 28.5% of the community born overseas and 16% from non-English speaking countries. Our employees proudly reflect the community in which they work. At Metro South Health 20.5% of staff identify as coming from non-English speaking backgrounds.

Having a workforce that reflects and understands the needs and expectations of our community is important to delivering safe, kinder, and more inclusive care.

We recognise our strength comes from the diversity of our people and so we encourage people of all genders, ethnicities, ages, abilities, languages, sexual orientation and family responsibilities to apply.

We are committed to creating a diverse and inclusive workplace for our people and our community.

Flexible working arrangements

Metro South Health supports and encourages work-life balance for the mutual benefit of Metro South Health and its employees. Work-life balance is about a person's ability to manage their paid work commitments with their career goals, personal, community and cultural responsibilities. Through flexible working arrangements such as leave, flex time accrual or a reduction in working hours, there is an opportunity to match the individual's requirements with those of the workplace – delivering quality health services.

Additional information

For further information about this opportunity and the benefits and conditions of working at Metro South Health please refer to the **Information for Applicants** document attached with this Smart Jobs advertised vacancy.

So you have finished your Role Description, what's next? Check out the Recruitment Process Flow for next steps, [here](#)



Patient Subpoena Export

Name: Mr Cory Lea Shepherd
Address: U104 158 Scarborough Street
Southport 4215
D.O.B.: 11/01/1991
Record No.: 46435
Phone:

Our Medical Ashmore
566 Olsen Avenue
Ashmore 4214
0756475000

Printed on 8th July 2025

Subpoena generated using the following date range: 01/01/2023 to 01/07/2024

Warnings:

Allergies/Adverse reactions:

Nil known.

Family History:

Mother: Not recorded

Father: Not recorded

Social History:

Occupation History:

Current Medications:

Medication	Dosage	Reason for Prescription	Reason for Cessation
Cialis 20mg Tablet	Use as directed.		
Fluoxetine 20mg Capsule	2 Capsules In the morning.		
Lomotil 2.5mg;0.025mg Tablet	1 prn up to tds for diarrhoea.		
Melatonin 2mg Tablet, modified release	1 Tablet Before bed.		
Valium 5mg Tablet	1 Tablet Daily p.r.n. Take for 60 Days.	Stress	
Viagra 100mg Tablet	1 Tablet Daily p.r.n.		

Reminders:

No reminders within range.

Actions:

No Actions within range.

Active Past History:

28/06/2023 Infection

Current

Inactive Past History:

Not recorded.

Immunisations:

None recorded.

Prescriptions:

Script date: 16/02/2023 Doctor: Dr W. Kamali
Drug: Viagra 100mg Tablet Dose: Tablet p.r.n.
Quantity: 12 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 16/02/2023 Doctor: Dr W. Kamali
Drug: Melatonin 1mg Tablet, modified release Dose: 1 Tablet Before bed
Quantity: 60 Repeats: 0 Script type: Non-PBS
Record Status: Current

Script date: 28/06/2023 Doctor: Dr S. Sathananthan
Drug: Roxithromycin 300mg Tablet Dose: 1 Tablet Daily
Quantity: 10 Repeats: 0 Script type: PBS Authority
Record Status: Current
Authority number: 26134571 Approval number: 10404
Authority indication: Infection

Script date: 12/09/2023 Doctor: Dr W. Kamali
Drug: Klacid 250mg Tablet Dose: 1 Tablet Twice a day
Quantity: 14 Repeats: 0 Script type: PBS
Record Status: Current

Script date: 12/09/2023 Doctor: Dr W. Kamali
Drug: Viagra 100mg Tablet Dose: Tablet p.r.n.
Quantity: 12 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 16/11/2023 Doctor: Dr P. Nanayakkara
Drug: Temaze 10mg Tablet Dose: 1 Tablet Before bed p.r.n.
Quantity: 25 Repeats: 0 Script type: PBS
Record Status: Current

Script date: 16/11/2023 Doctor: Dr P. Nanayakkara
Drug: Melatonin 5mg Tablet, modified release Dose: Tablet Before bed 6mg / nocte /prn --- Compounded
Quantity: 30 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 16/11/2023 Doctor: Dr P. Nanayakkara
Drug: Viagra 100mg Tablet Dose: Tablet p.r.n.
Quantity: 12 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 24/02/2024 Doctor: Dr W. Zhao
Drug: Proctosedyl 0.5%;0.5% Ointment Dose: 1 Application Three times a day
Quantity: 1x15g Repeats: 0 Script type: Non-PBS

Record Status: Current

Script date: 24/02/2024 Doctor: Dr W. Zhao
Drug: Viagra 100mg Tablet Dose: Tablet p.r.n.
Quantity: 12 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 24/02/2024 Doctor: Dr W. Zhao
Drug: Azithromycin 500mg Tablet Dose: 2 Tablet stat
Quantity: 2 Repeats: 0 Script type: PBS
Record Status: Current

Script date: 29/03/2024 Doctor: Dr S. Sathananthan
Drug: Viagra 100mg Tablet Dose: 1 Tablet Daily p.r.n.
Quantity: 12 Repeats: 5 Script type: Non-PBS
Record Status: Current

Script date: 16/05/2024 Doctor: Dr W. Zhao
Drug: Melatonin 5mg Tablet, modified release Dose: Tablet Before bed 6mg / nocte /prn --- Compounded
Quantity: 30 Repeats: 2 Script type: Non-PBS
Record Status: Current

Script date: 16/05/2024 Doctor: Dr W. Zhao
Drug: Viagra 100mg Tablet Dose: 1 Tablet Daily p.r.n.
Quantity: 12 Repeats: 5 Script type: Non-PBS
Record Status: Current

Script date: 28/06/2024 Doctor: Dr B. Slawinski
Drug: Melatonin 5mg Tablet, modified release Dose: Tablet Before bed 6mg / nocte /prn --- Compounded
Quantity: 30 Repeats: 2 Script type: Non-PBS
Record Status: Current

EPC Reports:

Investigation requests:

24/02/2024	Dr W. Zhao	Urine Chlamydia & Gono PCR First Pass	4Cyte Pathology
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Observations:

28/06/2023	Temp	37.2	Current	
24/02/2024	BP	138/85 Sitting	Current	3:54 pm
24/02/2024	Pulse	72	Current	3:54 pm

Consultations:

Surgery consultation

Recorded by: Dr Wali Kamali Visit date: 16/02/2023

Recorded on: 16/02/2023

for scripts
doing night shifts

also use viagra only when tired after night duties

was unable to get 5 mg melatonin

Actions:

Prescription printed: Melatonin 1mg Tablet, modified release 1 Before bed.

Prescription printed: Viagra 100mg Tablet p.r.n.

Surgery consultation

Recorded by: Dr Shivani Sathananthan Visit date: 28/06/2023

Recorded on: 28/06/2023

Having cold and flu symptoms

On sudafed, day and night tablets

cough medicine

1 week now, not improving

Examination:

General:

Temperature: 37.2

Throat inflamed

Nose congested

Chest clear

TM bilaterally dull

i/ URTI - bacterial

plan: oral abx

review if not improving

Diagnosis:

Infection

Reason for visit:

Infection

URTI, bacterial

Actions:

Prescription printed: Roximycin 300mg Tablet 1 Daily.

Roximycin 300mg Tablet ceased (No longer required).

Prescription printed: Roxithromycin 300mg Tablet 1 Daily.

Surgery consultation

Recorded by: Dr Wali Kamali Visit date: 03/07/2023

Recorded on: 03/07/2023

Came for medical cert

had URTRI

was seen here last week

did not get the medical cert

Reason for visit:

Medical certificate

Actions:

Letter printed.

Letter written re. 1. Medical Certificate Ashmore.

Surgery consultation

Recorded by: Dr Wali Kamali Visit date: 12/09/2023

Recorded on: 12/09/2023

CO sorethroat for few days
fever on & off

Examination:

temp 37.5

Ear, nose & throat:

No nasal discharge. No nasal obstruction. Red throat. Both tonsils enlarged. No pus on tonsils.

Reason for visit:

Sore throat

Actions:

Prescription printed: Klacid 250mg Tablet 1 Twice a day.

Prescription printed: Viagra 100mg Tablet p.r.n.

Surgery consultation

Recorded by: Dr Priyal De Silva Nanayakkara Visit date: 16/11/2023

Recorded on: 16/11/2023

h/o

Poor sleep

Shift work

Takes melatonin 2 mg to help to go to sleep, soem time 4 mg, but does not help much

Cannot work/ do shifts if he does not get a good sleep.

Does not take them every day

No psychological illness such as depression/ psychosis.

mood good.

I did a script for Melatonin- 6mg(get from Compounding chemist)

Temaze prn only

Review interval:

progress.

Actions:

Dose of changed from to Before bed 6mg / nocte /prn --- Compounded..

Prescription printed: Temaze 10mg Tablet 1 Before bed p.r.n.

Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.

Prescription printed: Viagra 100mg Tablet p.r.n.

Surgery consultation

Recorded by: Dr Wali Kamali Visit date: 24/11/2023

Recorded on: 24/11/2023

For medical form

applying for his super

having multiple dental procedures

ad should ask the sp tpo do the form

pt pt not understanding at all

kepp asking to fill the form

form done

Surgery consultation**Recorded by: Dr William Zhao Visit date: 24/02/2024****Recorded on: 24/02/2024**

renew script for viagra
responded well, nil adverse reaction

developed haemorrhoids during his recent gym workout

had unprotected sex with a person few weeks ago, who was diagnosed with chlamydia
he is asymptomatic

Examination:**General:**

BP (sitting): 138/85

Pulse: 72

Actions:

Prescription printed: Proctosedyl 0.5%;0.5% Ointment 1 Three times a day.
Prescription printed: Viagra 100mg Tablet p.r.n.
Request printed to 4Cyte Pathology: Urine Chlamydia & Gono PCR First Pass.
Prescription printed: Azithromycin 500mg Tablet 2 stat.

Surgery consultation**Recorded by: Dr Shivani Sathananthan Visit date: 29/03/2024****Recorded on: 29/03/2024**

Presents for script of viagra
Lost his repeats

Reason for visit:

Erectile dysfunction

Actions:

Dose of Viagra 100mg Tablet changed from p.r.n to 1 Daily p.r.n.
ePrescription sent to coryshepherd1@hotmail.com: Viagra 100mg Tablet 1 Daily p.r.n.

Surgery consultation**Recorded by: Dr William Zhao Visit date: 16/05/2024****Recorded on: 16/05/2024**

renew referral to psychiatrist

renew script for Viagra
lost his script
understands the maximum dose is one a day
warned risk of OD

Actions:

Letter to Dr Arash Amini printed.
Letter written to Dr Arash Amini re. 1.Specialist Referral Letter Ashmore.
Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.
Prescription printed: Viagra 100mg Tablet 1 Daily p.r.n.

Surgery consultation**Recorded by: Dr Bogdan Slawinski Visit date: 28/06/2024****Recorded on: 28/06/2024**

wants melatonin
to help to sleep

stress at work
upset by people not following rules

Examination:

well, alert, t 36,6

Reason for visit:

Anxiety

Actions:

Prescription printed: Melatonin 5mg Tablet, modified release Before bed 6mg / nocte /prn --- Compounded.

Letter printed.

Letter written re. 1. Medical Certificate Ashmore.

Surgery consultation

Recorded by: Dr Peter Hawes Visit date: 01/07/2024

Recorded on: 01/07/2024

work problems- ethical complaint about manager and director, they don't follow the law

been there 5 years

feels he is undervalued

they withhold pay at times, no overtime- not processed, manipulate his roster- so he works lates then earlies, not allowed to do this

they don't listen to his complaints

all this is stressing him out, causing anxiety,

Reason for visit:

work stress

Actions:

Letter printed.

Letter written re. QLD Work capacity certificate.

Investigations:

ED Information for Patient (Verified)

Gold Coast University Hospital
Emergency Department
1 Hospital Boulevard, Southport, QLD 4215
DISCHARGE LETTER
CONFIDENTIAL

06-Jun-2024 18:11:35
SHEPHERD, CORY
104/1058 SCARBOROUGH STR SOUTHPORT QLD 4215
DOB: 11-Jan-1991
URN: GCH 432749
Dear Treating Doctor,

SHEPHERD, CORY attended the Emergency Department at Gold Coast University Hospital on the
06-Jun-2024 12:30:04.

The diagnosis was Chronic dental pain.

Hospital Course:

Cory presented to ED with dental/gum pain. BG of multiple max facs surgeries related to an old tooth extraction of tooth 13. Three bone grafts and insertion of plate and mesh. Surgeon had planned to remove the mesh last year however, Cory did not have the funds for it and had minimal symptoms at that time so has not gone through with it yet.

For the past 4 weeks he has had increasing pain to his gum which is now radiating up into his cheek and orbit. Systemically well. Area of mesh now exposed through his gum.

Case was discussed with the maxfac team who recommended a CT scan tonight and then follow up in their clinic. Cory did not wish to wait in ED for his CT scan as he has had a long wait in ED already and has elected to go and see his GP for a CT scan of his face with contrast. He was commenced on PO augmentin and advised to use a chlorhexidine mouthwash. Given panadeine forte for analgesia.

Discharge Instructions:

Investigation results are available either through the Health Provider Portal or MyHealth Record. Please see your General Practitioner to access your Results.

Medication Instructions:

(Clinician Signature) _____
Clinician Name and Designation:

*For any concerns after discharge, please see your GP or return to the Emergency Department ASAP.

Information requests from GPs: Please fax letter head with patient full name, date of birth and detailed description of the information required to (07) 5668 6952.



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | **Phone** 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

A Cornerstone Health Pty Ltd Medical centre

Operated by

Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

MEDICAL CERTIFICATE

03/07/2023

THIS IS TO CERTIFY THAT :

Mr Cory Lea Shepherd has a medical condition and will be unfit for Work/ Study/ Usual activity
from 24/06/2023 to 02/07/2023 inclusive.

Dr Wali Kamali
MBBS, FRACGP, AFRCS, FRNZCGP
2341138Y



Our Medical Ashmore

566 Olsen Avenue, Ashmore QLD 4214

HealthLink ourmedic | **Phone** 07 5647 5000

Fax 07 5616 8014

Email reception@ourmedicalashmore.com.au

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16/05/2024

Dr Arash Amini

Evandale Practice

Suite 2, 1st Floor Evandale Place, 142 Bundall Road

Bundall 4217

Phone: (07) 5510 3122

Fax: (07) 5510 3144

Re: Mr Cory Lea Shepherd

Add: U207 W\r\n1 Marina Drive

Benowa 4217

DOB: 11/01/1991

Medicare No: 4330920095 1

Ph: 0422438627

Dear Arash,

Thank you for seeing Mr Cory Lea Shepherd for ongoing care and management.

Current medications are:

Azithromycin 500mg Tablet

Melatonin 5mg Tablet, modified release

Viagra 100mg Tablet

2 Tablets stat.

Before bed 6mg / nocte /prn --- Compounded.

1 Tablet Daily p.r.n.

Past Medical History:

26/10/2022	ADHD
26/10/2022	Anxiety
28/06/2023	Infection

Allergies:

Nil known.

Yours faithfully,

Dr William Zhao
BMED,AMC, FRACGP
Provider No: 264389EW



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Wycombe Services Pty Ltd

ABN 40 613103 450

www.ourmedicalhome.com.au

MEDICAL CERTIFICATE

28/06/2024

THIS IS TO CERTIFY THAT :

Mr Cory Lea Shepherd has a medical condition and will be unfit for Work/ Study/ Usual activity
from 27/06/2024 to 11/07/2024 inclusive.

Dr Bogdan Slawinski
General Practitioner, MBBS, FACEM
208300QJ

Work capacity certificate – workers' compensation

Form 132M — Version 1

Workers' Compensation and Rehabilitation Act 2003

IMPORTANT INFORMATION: Work is an important part of recovery. In most cases an early return to work (or remaining at work) is beneficial for health and wellbeing. The treating practitioner's guidance increases the likelihood of positive return to work outcomes. A worker receiving continued support is three times more likely to regain their capacity to work. Consider the health benefits of work when certifying the patient's capacity.

Part A - Patient details

Name Cory Shepherd Date of Birth 11/01/1991
Mobile number 0422438627 Claim number S23LW142013 ☒ New claim ☐ Claim is report only
Occupation (if known) Admin worker Patient's employer Qld Health

Part B - Injury details

Date of examination 01/07/2024 Patient's stated date of injury 18/06/2024 Patient was first seen at this practice/hospital for this injury/disease on 01/07/2024

The patient is/was suffering from (List all work-related diagnoses. *If symptoms only, tick "Provisional diagnosis"*) ☐ Provisional diagnosis anxiety, stress

Patient's stated mechanism of injury harrasment at work

Is this consistent with your clinical findings?

☒ Yes ☐ Unclear

Describe mechanism in detail ongoing breaking of workplace rules by bosses, victimizing him
Pre-existing factors or condition
aggravated (if not previously supplied)

Part C - Treatment plan

Patient requires/d treatment from 01/07/2024 to 04/08/2024 to be reviewed again on 04/08/2024 ☐ No further review

Treatment needs time way from this environment

I have prescribed medication that may impede safe work, travel or cognitive function ☐ No ☐ Yes

Referrals ☐ Diagnostic ☐ Allied Health ☐ Name/discipline Details (specify)
Specialist/GP

Part D - Capacity for work (choose from one of three options)

☐ The certified injury does not prevent a return to pre-injury duties. Do not complete part E. Go to Part F. ☐ If suitable duties available, can return to some form of work from ☒ No functional capacity for any type of work until

Complete the below section if you certified no functional capacity for any type of work

If no functional capacity, state why? (if no capacity for more than 7 days, the insurer may contact you to obtain more information)
stressed and anxious,

Estimated time to return to some form of work duties unknown

Estimated time to return to full duties unknown

Part E - Functional ability

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

No change since last certificate

Certification should be based on what CAN be done, NOT available duties. Consider what the patient can do, either at work or home.

Function/task (patient's usual functional ability)	Is functional ability affected by injury/condition?	Note any restrictions (if relevant)	What patient can do (if "yes" is selected)
Lower limb	No		
Upper limb	No		
Hand function	No		
Spinal function	No		
Cognition/psychosocial functioning	No		
Driving a car	No		
Operating machinery/heavy vehicle	No		
Manual tasks	No		
Other	No		

Part F - Rehabilitation at work - return to work plan

(Optional for emergency medical practitioners/dental practitioners. Nurse practitioners not to complete.)

What workplace modifications are required to facilitate return to work? (e.g. work site assessment, psychosocial considerations)

Other considerations or factors that may affect recovery (the insurer can arrange appropriate support)

☐ I require a suitable duties program to be provided to me for approval

I have discussed injury requirements and return to work with the patient and ☐ Employer ☐ Insurer ☐ Rehabilitation provider

Part G - Medical/dental/nurse practitioner details and statement (or use practice/hospital stamp)

I have discussed the information contained in this certificate with the patient. I have provided the clinical information in this certificate.

Name Peter Hawes

Email admin@ourmedicalashmore.com.au

Practice/hospital Our Medical Ashmore

Phone 0756475000

Date 01/07/2024

Postal Address

Signature

566 Olsen Avenue, Ashmore

QLD 4214

Further information www.worksafe.qld.gov.au/medicalsupport

All enquiries (medical/dental/nurse practitioner, patient, employer) 1300 362 128

Under the Workers' Compensation and Rehabilitation Act 2003 (the Act), the workers' compensation insurer is authorised to collect the information on this form to process the claimant's application for compensation. The information contained in this form may be disclosed to the claimant's employer, another insurer, medical or allied health providers or any other workers' compensation authority in any jurisdiction. The claimant may be contacted by the insurer, and the insurer may contact the claimant's employer and any other medical, allied health or rehabilitation provider about the injury. This form was approved by the Workers' Compensation Regulator on 31 May 2016, pursuant to section

588 of the Act. © State of Queensland (Queensland Treasury) 2016



Employee Capability Checklist – Cory Shepherd

*** Please note that a separate medical certificate is also required to support the completed Capability Checklist.*

Information provided in the Capability Checklist below will assist our Organisation to determine appropriate tasks in the event that the identified worker is cleared for suitable duties. Please ensure that all sections of the Capability Checklist are completed in full.

I have examined Cory Lea Shepherd (DOB 11 January 1991) and consider that they are diagnosed with the following condition:

Major Depressive Disorder with anxious distress (DSM-5 296.23) — work-related psychological injury, onset 18 June 2024.

- ☒ I have viewed the worker's Position Description (attached — Administration Officer, Switchboard Services (AO3), Logan Hospital) and/or
- ☒ I am aware of the requirements of their role

It is my opinion that they are fit to resume:

- ☐ Normal duties
- ☒ Modified duties
- ☒ Reduced hours (hours/days)
- ☐ Workplace Performance Discussions

Modified duties relate to:

- ☒ Tasks performed
- ☒ Duration

Hours to be worked

Week 1: 8 hours/day 3 days/week	Week 5: 8 hours/day 3 days/week
Week 2: 8 hours/day 3 days/week	Week 6: 8 hours/day 3 days/week
Week 3: 8 hours/day 3 days/week	Week 7: 8 hours/day 3 days/week
Week 4: 8 hours/day 3 days/week	Week 8: 8 hours/day 3 days/week
(6 shifts per 10-shift fortnight: max 2 weekday day/afternoon + 2 night + 2 weekend shifts — continuation of the reduced pattern Mr Shepherd has sustained since 2025)	

Fit to commence the above from: 3 July 2026 (continuation of existing arrangement)

Date of next review: 28 August 2026 (approximately 8 weeks)

Activity	No capacity	Restricted / As Tolerated	Normal	Details of any restrictions and reasons
COGNITIVE				
Learning new tasks	☐	☐	☒	
Ability to initiate tasks independently	☐	☐	☒	
Understanding instructions	☐	☐	☒	
Carrying out instructions accurately	☐	☐	☒	
Maintain focus/concentration	☐	☒	☐	Concentration reduced during sustained high-stress periods; managed by rostering to lower call-volume shifts.
Executing tasks within a reasonable timeframe	☐	☐	☒	
Working autonomously without supervision	☐	☐	☒	
Prioritising tasks	☐	☐	☒	
Making decisions	☐	☐	☒	
Multi-tasking	☐	☒	☐	Concurrent high-volume demand exacerbates symptoms; suitable on lower-volume shifts.
Memorising information	☐	☒	☐	Working memory affected under stress (treating psychiatrist report 13-Feb-2025).
Working in a high demand environment	☐	☒	☐	Limit weekday day/afternoon shifts — which carry the peak call volume and the majority of complaints — to no more than 2 per fortnight; remaining shifts rostered to the lower-volume weekend (2) and night (2) shifts.
RELATIONAL				
Communicating with internal/external customers	☐	☐	☒	
Working in a team environment	☐	☐	☒	
Handling conflict	☐	☒	☐	No complaint-handling duties: complaints received at the Switchboard are to be logged and redirected to management/the dedicated complaints area, not actioned, resolved or absorbed by Mr Shepherd.
Supervising others	☐	☐	☒	Not a requirement of the AO3 role.
Interacting appropriately with others	☐	☐	☒	
Do you foresee any existing stressors that could impact on a return to substantive role? If so, please provide additional comments:				Yes. (1) Complaint-handling exposure at the Switchboard; (2) being held accountable, and blamed, for failures that are the responsibility of others across the hospital, regardless of where the fault lies; (3) rostering and fatigue-management practices; (4) unresolved workplace matters involving line management. Recommend return-to-work communications and this checklist be managed through the Injury Management Team Human Resources.

Medications

Is the worker currently taking any medication?

☒ Yes

☐ No

Please outline any side effects from medication currently being taken that may impact on a person's functional capacity or task performance (for example drowsiness, dizziness, confusion, impaired concentration, fatigue, slurred speech):

Current medications:

1. Fluoxetine 60 mg mane (3 × 20 mg capsules).
2. Lisdexamfetamine (Vyvanse) 30 mg mane — for cognitive symptoms and focus.

Quetiapine (Seroquel) 25 mg nocte, previously prescribed for rumination and sleep, has been ceased.

Relevant effects on task performance: maintaining a regular sleep routine remains central to symptom management following cessation of the night-time medication. This supports (a) limiting night shifts to no more than 2 per fortnight, (b) predictable rostering published in advance, and (c) a minimum 10-hour break between rostered shifts consistent with the applicable Award and fatigue risk management standards. Residual symptoms include reduced concentration and working memory under sustained high-demand load and fatigue sensitivity.

Additional Comments:

1. Please elaborate on the current medical condition Mr Shepherd is suffering from?

Major Depressive Disorder with anxious distress (DSM-5 296.23), a work-related psychological injury with onset 18 June 2024. Treatment is ongoing (fluoxetine, lisdexamfetamine) under psychiatric care. Residual symptoms most relevant to work capacity are reduced concentration and working memory under sustained high-demand load, fatigue sensitivity, and symptom exacerbation on exposure to the identified workplace stressors (complaint handling; being held accountable and blamed for the failures of others; unpredictable rostering).

2. In your opinion do you feel that Mr Shepherd's current condition is temporary, episodic or permanent in nature? What is the expected duration of the current restrictions?

Episodic, with symptom exacerbation linked to stressor exposure. The restrictions below are expected to remain necessary for at least the next 3–6 months, subject to treatment response and 8-weekly review.

3. Based on your current assessment, what work capacity (including number of shifts, hours, and type of duties) would best support Mr Shepherd's recovery while minimising risk of exacerbation?

Six 8-hour shifts per fortnight (of the ten rostered in a full-time fortnight), comprising:

(a) No more than two weekday day/afternoon shifts. These shifts carry the peak call volume and the majority of complaints — the identified stressors. Retaining two per fortnight maintains graded exposure to the full role and preserves work capacity; the cap prevents multiple high-load shifts accumulating within a fortnight, which is the pattern associated with symptom exacerbation.

(b) Two weekend day shifts. Weekend call volume and complaint exposure are materially lower while the duties are identical, allowing the same work to be performed at an intensity consistent with Mr Shepherd's current concentration and working-memory capacity, and preserving a normal sleep-wake cycle.

(c) No more than two night shifts at this stage. Night shifts are low in volume and complaints, but a stable sleep routine is central to symptom management following cessation of the night-time medication; capping nights protects that routine and limits cumulative fatigue. This cap is suitable for reassessment at review.

This reduced pattern of approximately six shifts per fortnight is the pattern Mr Shepherd has in fact worked and tolerated over the past twelve months without deterioration, and maintaining rather than increasing it best supports recovery while minimising risk of exacerbation. Usual switchboard operational duties remain suitable; complaint-handling duties are excluded as below.

4. Are there any specific workplace modifications or supports you would recommend (e.g. reduced workload, predictable rostering, supervision level, reduced exposure to stressors) to assist Mr Shepherd during this period?

(a) No complaint-handling duties — complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd; (b) predictable rostering published in advance, with a minimum 10-hour break between shifts consistent with the Award and fatigue risk management standards; (c) return-to-work coordination and correspondence through the Injury Management team, Human Resources; (d) access to EAP (TELUS Health) as an available support; (e) review in approximately 8 weeks.

Please complete this form and return to the Injury Management team, Logan & Beaudesert Health Service, Human Resources by email lbh_injurymanagement@health.qld.gov.au

Please include your practice details/stamp here

Signature

Date



3-7-26.

**MY DOCTORS CLINIC
OPEN 24 HOURS**

Dr Day Hong Ma

MBBS,FRACGP,FACRRM Provider No: 4696842K

16/3221 Surfers Paradise Blvd

Surfers Paradise, QLD 4217

Ph: 07-5592 2299 Fax: 07-5539 9188

www.mydoctorsclinicsurfers.com.au

Attachment 5 — movement forms, 2026

Written on each form.

Written on the form	Form 1	Form 2	Form 3
Delegate	Scott Hughes	Scott Hughes	Scott Hughes
Delegate approval	27.02.2026	17.04.2026	09.06.2026
Effective period	01.03.2026 – 15.03.2026	16.03.2026 – 26.04.2026	25.05.2026 – 28.06.2026
Fortnightly hours	56.00	56.00	40.00
Shift arrangements	Continuous Shift Worker	Continuous Shift Worker	Continuous Shift Worker
Position	AO Switchboard, AO3, Level 04	AO Switchboard, AO3, Level 04	AO Switchboard, AO3, Level 04

The three forms follow.

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 01.03.2026 - 15.03.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd to extend reduce hours contract - 1.03.26 - 15.03.26 possible extension

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

19.02.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**26.02.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna**27.02.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

27.02.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 16.03.2026 - 26.04.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd reduced hours from 1FTE to 0.7fte per fortnight 16.03.2026 - 26.04.2026 with possible extension after review.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

09.04.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**13.04.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna**15.04.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:
Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

17.04.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 25.05.2026 - 28.06.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: AO3
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 40.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd request to reduce contracted hours from 1.0FTE - 0.5TE 40hrs per fortnight.
Temp request until 28.06.26 until further review and documentation to be provided to support request.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

27.05.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**05.06.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Melissa Spink**08.06.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

09.06.2026

Attachment 7 — request for medical information, 31 July 2026

WC/2024/227 · Shepherd v Workers' Compensation Regulator · two documents of the employer, reproduced in full and unmarked.

Written on the documents	
Document 1	Letter to Mr Cory Shepherd — the request for medical information (2 pages)
Document 2	Letter to Dr Day Hong Ma with the nine-question schedule — <i>“Request for Medical Information”</i> (5 pages)
Signed by	Mr Scott Hughes, Director, Corporate Services, Logan Beaudesert Health Service, Metro South Health — on each document
Date	31 July 2026
Made under	HR Policy G3: Reasonable Adjustment; sections 17 and 19 of the <i>Work Health and Safety Act 2011</i> — as stated in Document 1

The two documents follow.

Enquiries to: Michelle Harrison
Injury Management Consultant
Human Resources
Telephone: 07 3542 2744



Metro South Health

Logan and Beaudesert Health Service

Mr Cory Shepherd
12304/3113 Surfers Paradise Blvd
SURFERS PARADISE QLD 4217

Dear Mr Shepherd

I refer to your employment as an Administration Officer (**AO**), Switchboard Operations, Corporate Services, Logan Hospital (**LH**), Metro South Health (**MSH**) on a Permanent Full-time basis 76 hours per fortnight and your recently completed Employee Capabilities Checklist (**ECC**) (**Attachment 1**) completed by Dr Day Hong Ma on the **3 July 2026** which was provided to us to facilitate a Return to Work Plan.

I am seeking further clarification from Dr Day Hong Ma, to understand the nature of some of the restrictions stated in the ECC and how this relates to your current medical condition/s and the effect they have on your ability to perform the inherent requirements of your role Administration Officer (**AO**), Switchboard Operations, Corporate Services (**CS**) Logan Hospital.

I require further information from your general practitioner to confirm:

- The impact of your medical condition and/or disorder on your ability to perform your role;
- Any risks to your health and safety in undertaking the role;
- Any other matters that are considered relevant to your medical condition and/or disorder.

You may choose to:

1. Obtain this information from your medical practitioner on the attached '*Request for Medical Information*'; or
2. Provide Metro South Health permission to obtain this information directly from your medical practitioner. An authority form is attached should you prefer this option. Please let me know if you prefer this option.

We are requesting this information in accordance with the provisions of the Human Resources (**HR**) Policy G3: Reasonable Adjustment, and sections 17 and 19 of the *Work Health and Safety Act 2011*. This information will primarily be used for the purposes of better understanding your condition and to support the implementation for appropriate controls within the working environment or any other reasonable adjustments.

I ask that you please return the applicable document within **seven (7) calendar days** of receipt of this letter. This will enable me to give proper consideration to your requested restrictions to duties and determine what accommodations we can provide. If you do not return the document within this timeframe or seek a reasonable extension, I will make a decision regarding your ability to perform your role based on the available information.

Better together

MSH will meet your reasonable costs of preparing this report and we appreciate your assistance with this request. Please send invoices to **Michelle Harrison**, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Services via email at lbh.InjuryManagement@health.qld.gov.au.

Employee support

I acknowledge this may be a difficult time for you. I would like to take this opportunity to also advise you of the Employee Assistance Program. This is a confidential counselling service which is free of charge to all employees of MSH for up to six sessions per calendar year. Access to this service is by self-referral.

Should you wish to access Employee Assistance, please contact TELUS Health on 1800 604 640, or for Aboriginal and Torres Strait Islander support, please contact TELUS Health on 1800 816 152.

More information about the support TELUS Health can provide can be found out their website <https://app.one.telushealth.com/> or by downloading the free TELUS Health One app. You can log on to both using the following credentials:

Username: mshhs
Password: mshhs

Questions

If you have any questions in relation this correspondence or the process, please contact **Michelle Harrison** Injury Management Consultant, Human Resources, Logan and Beaudesert Health Service via email at lbh.InjuryManagement@health.qld.gov.au or Ph: (07) 3542 2744.

Yours sincerely,



Scott Hughes
Director, Corporate Services
Logan Beaudesert Health Service
Metro South Health
31/07/2026

Enquiries to: Michelle Harrison
Injury Management Consultant
Human Resources
Telephone: 07 3542 2744



Metro South Health

Logan and Beaudesert Health Service

Dr Day Hong Ma
My Doctors Clinic
16/3221 Surfers Paradise Boulevard
SURFERS PARADISE QLD 4217

Dear Dr Day Hong Ma,

Name of Employee: Mr Cory Shepherd
Date of Birth: 11 January 1991

I refer to your patient, Mr Cory Shepherd and his employment as an Administration Officer (**AO**), Switchboard Operations, Corporate Services (**CS**), Logan Hospital (**LH**), Metro South Health (**the Health Service**) on a Permanent Full-time bases (76 hours per fortnight).

Background

Logan and Beaudesert Health Service (**LBHS**) is in receipt of an Employee Capabilities Checklist (**ECC**) (**Attachment 1**) completed by you on the 3 July 2026.

In accordance with Queensland Health Human Resource (**HR**) Policy G3: *Reasonable Adjustment*, and sections 17 and 19 of the *Work Health and Safety Act 2011*, the Health Service is seeking further clarification of the medical information you have provided regarding Mr Shepherd. The purposes of this request is to better understand Mr Shepherd's condition and current capacity for work, including any restrictions or limitations that may need to be considered.

This information will assist the Health Service to identify and implement any appropriate workplace controls or reasonable adjustments to support a safe and sustainable return to his substantive position.

Questions

I appreciate your assistance with this request. Should you require any additional information or have a query about this request please contact Michelle Harrison, Injury Management Consultant, Human Resources, Logan and Beaudesert Health Service via email at lbh_injurymanagement@health.qld.gov.au or phone: (07) 3542 2744.

Yours sincerely

Scott Hughes
Director, Corporate Services
Logan Beaudesert Health Service
Metro South Health
31/07/2026

Attachment 1 – Employee Capabilities Checklist
Attachment 2 – Role Description

Better together

Information requested from Dr Day Hong Ma:

1. The attached Employee Capabilities Checklist states that Mr Shepherd has a diagnosis of Major Depressive Disorder (**MDD**) with anxious distress, with symptoms exacerbated on exposure to identified workplace stressors, including complaint handling, being held accountable and blamed for the failures of others, and unpredictable rostering.

Could you please clarify the below question in relation to this statement:

- a. When was Mr Shepherd first diagnosed with MDD?

- b. What is the clinical basis for identifying these specific workplace stressors as contributing to the exacerbation of Mr Shepherd's condition.

- c. Whether these workplace stressors are based solely on Mr Shepherd's self-report, your own clinical assessment, or other medical information/reports available to you.

- d. Whether exposure to these workplace stressors presents a foreseeable risk to Mr Shepherd's health or safety if he were to return to work, and if so, what specific workplace controls or adjustments you consider medically necessary to manage that risk.

2. You have described Mr Shepherd's condition as a "*work-related psychological injury*", with existing stressors that could impact on a return to substantive role, specifically:
- "(2) *being held accountable and blamed for failures of others*".
 - "(4) *unresolved workplace matters involving line management*".

As the Health Service is not aware of any concerns being raised for appropriate management, clarification on these statements is being sought to better understand the nature of his injury/illness, including:

- If Mr Shepherd is able to follow a reasonable and lawful direction is issued by his supervisor.

- Participate in discussions in relation to his workplace performance and/or conduct.

Please provide any further context and/or considerations relating to Mr Shepherd's *work-related psychological injury*?

3. Having regard to your reference to unresolved workplace matters involving line management, please confirm whether Mr Shepherd is medically fit to return to their substantive role under the existing reporting arrangements, including working with and reporting to their current line manager.

4. If Mr Shepherd is not able to return to his substantive role under the existing reporting arrangements, please specify any restrictions or adjustments you recommend, the clinical basis for those recommendations in functional terms, and the anticipated duration and review date.

5. The ECC recommends that Mr Shepherd not undertake complaint-handling duties, can you please clarify what specific activities are encompassed within “complaint handling” does this restriction relate to receiving complaints, documenting complaints, redirecting complaints, resolving complaints, or all complaint-related interactions.

For consideration in responding to this question, established process for client complaints received by Switchboard employees is for immediate escalation and management by the Health Service Client Liaison Officer and/or Manager, Switchboard Services.

6. In your medical opinion is Mr Shepherd able to fulfill the full inherent requirements of his Role Administration Officer – Switchboard Services (**Attachment 2**) *without* restrictions or modifications to duties?

7. Your report states Mr Shepherds “working memory is affected under stress”. Could you please clarify what this means in functional terms in the workplace, including:
- a. how this may affect their capacity to perform their substantive duties under normal workplace demands;
 - b. the types of circumstances likely to cause this difficulty;

8. Are there specific tasks, situations, or environments (e.g., high-pressure settings, shift work, patient acuity) that may exacerbate Mr Shepherd’s condition or symptoms?

9. If we are not able to accommodate the restrictions you have recommended, is Mr Shepherd able to safely return to the workplace?

Please include your practice details/stamp here

Signature

Date